

2009

PHARMACEUTICAL CARE

Where do we stand -
Where should we go ?



European Directorate for the
Quality of Medicines & HealthCare

Pharmaceutical Care: Where Do We Stand – Where Should We Go?

Key Concepts in Pharmaceutical Care, Quality Assessment of Pharmaceutical Care in Europe, Sources of Information

Survey Report

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2009

The views expressed in this report do not necessarily reflect the official opinions of the Council of Europe and its Europe Directorate for the Quality of Medicines & HealthCare.

Sincere thanks are owed to the members of the Committee of Experts on quality and safety standards in pharmaceutical practices and pharmaceutical care (CD-P-PH/PC) “Quality of Care” working party for their great support and valuable input throughout the carrying out of the survey and its evaluation. In particular, the study authors wish to thank Prof. Marcel Bouvy, PhD PharmD, Consultant, and Dr Martine Kruijtbosch, both from SIR Institute for Pharmacy Practice and Policy, for analysing and compiling the survey data and for their affable co-operation throughout the project.

The study authors wish to honour greatly the contribution of all survey respondents, national public health authorities, national doctors’, pharmacists’, nurses, patients’ and other stakeholders’ associations, for the time and effort they have devoted to replying to the questions and for sharing openly and constructively their views on pharmaceutical care in Europe.

The study authors trust that the study conclusions and recommendations will be of use to those health policy-makers and health professionals actively engaged in improving the quality of pharmaceutical care in Europe.

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1. BACKGROUND

Since long the Council of Europe is concerned with pharmaceutical care: Committee of Ministers Resolution ResAP(93)1 on the role and training of community pharmacists¹ was inspired by the the principles of patient-centred multidisciplinary pharmaceutical care and called member states to take these principles into account when regulating the role and training of community pharmacists.

In its report on the “Creation of a better medication safety culture in Europe: Building up safe medication practices” (2006), the former Expert Group on Safe Medication Practices (P-SP-PH/SAFE) emphasised the importance of a patient-centred approach in order to increase medication safety [1].

The report stressed that activities, co-ordinated throughout Europe, aimed at promoting multi-professional co-operation and knowledge, skills, attitudes and values in practices and care involving pharmaceuticals would promote the safe and effective use of medicines. Quality indicators could be a means of:

- helping to increase awareness and provide practical guidance on improving the quality of pharmaceutical practice and care throughout Europe;
- improving pharmaceutical care and practice;
- preparing proposals for harmonised provisions and practices involving pharmaceuticals in states in Europe in the field of safety in practice and care involving pharmaceuticals.

The first step of this process is to study current awareness of the concept of pharmaceutical care among healthcare professionals and other stakeholders, to the extent to which pharmaceutical care is implemented in daily practice, and the competences and skills required.

Activities co-ordinated by the European Directorate for the Quality of Medicines and HealthCare (EDQM)

The Committee of Experts on quality and safety standards in pharmaceutical practices and pharmaceutical care (CD-P-PH/PC) of the Council of Europe - European Directorate for the Quality of Medicines & HealthCare (EDQM) was called upon “... to improve public healthcare and practices involving pharmaceuticals in community, ambulatory care, primary care, hospitals and nursing homes through specific programmes and policies, putting first the needs of patients and society in general, valuing the social and ethical context of healthcare ...” (terms of reference).

To achieve this, it is expected to “... develop and carry out a programme of activities aiming at improving public healthcare in Europe through promoting multi-professional co-operation and knowledge, skills, attitudes and values in practices and care involving pharmaceuticals, in particular:

- carrying out studies on the implementation of quality assessment in pharmaceutical practice and care in Europe through quality indicators, preparing proposals for harmonised provisions and practices involving pharmaceuticals in member states in the field of safety in practice and care involving pharmaceuticals;
- contributing to the practical implementation of the above provisions and policies through programmes, raising awareness and providing practical guidance ...”.

The European Committee on Pharmaceuticals and Pharmaceutical Care (CD-P-PH) (steering body) asked the CD-P-PH Committee of Experts to prepare by the end of March 2009 a preliminary report on the project on the “Development and implementation of indicators for the documentation (assessment) of the quality of pharmaceutical care in Europe taking account of the new roles of the pharmacist”. The report was intended to indicate the position of:

- the member states’ definitions of key concepts in pharmaceutical care;
- the performance indicators used in member states to evaluate good quality of pharmaceutical care and pharmaceutical services;
- the relevance of existing databases;
- the relevant stakeholders involved in pharmaceutical care at the national level. The CD-P-PH/PC Committee of Experts therefore carried out a survey in Council of Europe member states, both EU members and non-members on “The definitions of key concepts in pharmaceutical care, indicators for good quality of pharmaceutical care and services, existing sources for relevant data/information in the states parties to the Convention on the Elaboration of a European Pharmacopoeia (Ph. Eur.), observers to the Ph. Eur., and other Council of Europe member states”.

The draft survey questionnaire was developed by the CD-P-PH/PC Committee of Experts working party on “Quality of Pharmaceutical Care”. The questionnaire was sent to **147** key national stakeholders in pharmaceutical care identified by the delegations on the CD-P-PH Committee, steering body. Nineteen CD-P-PH delegations from eleven EU and eight non-EU member states² sent contact details of target respondents, belonging to national public health

¹ Committee of Ministers Resolution ResAP(93)1 on the role and training of the community pharmacists. <https://wcd.coe.int/com.instranet.InstraServlet?command=com.instranet.CmdBlobGet&InstranetImage=594151&SecMode=1&DocId=1162386&Usage=2>

² Austria, Belgium, Bosnia and Herzegovina: Federation of Bosnia and Herzegovina and Republika Srpska, Cyprus, Czech Republic, Hungary, Iceland, Ireland, Italy, Latvia, Macedonia, Montenegro, the Netherlands, Norway, Portugal, Switzerland, Turkey, and the United Kingdom

authorities, national doctors', pharmacists', nurses' and patients' associations and other national stakeholders. The survey was sent to experts (doctors, nurses and pharmacists) from the former Expert Group on Safe Medication Practices (P-SP-PH/SAFE), Council of Europe former Partial Agreement in the Social and Public Health Field.

The findings of the survey were discussed by the authors, with reference to the scientific publications on pharmaceutical care in Europe. The replies from the experts from the former P-SP-PH/SAFE group of experts were analysed separately.

The survey report includes a summary and key trends in the survey information, recommendations for the development of a specific programme of activities, and a detailed discussion of the individual survey replies. Examples of recommended activities include the development of indicators for the quality of pharmaceutical care in ambulatory and clinical settings from a multi-professional perspective, including patients' participation.

2. RESULTS

The methods, recipients and respondents, and reply rates are presented in chapters 5-7. The survey results are discussed in detail in chapters 5-7.

Fifty-eight (58) replies from national public health authorities, national doctors', pharmacists', nurses, patients' associations, and other stakeholders from 17 countries³ provided sufficient information for analysis and conclusions. The respondents were from Council of Europe member states, both EU members and non-members, in the major geographical regions. The respondents' countries may be considered representative of the Council of Europe's membership (see chapter 6).

Merely by indicating representatives in national public health authorities, national doctors', pharmacists', nurses/carers', patients', disabled persons' and social workers' associations for participation in the survey, the delegations of the steering body provided valuable information on relevant stakeholders in pharmaceutical care at the national level.

Although the replies from pharmacists' associations represented a large proportion of the replies (29.3%), the reply rates of national public health authorities, patients' associations and other healthcare associations (doctors and nurses) were sufficient in number and quality to draw conclusions on essential questions of the survey⁴. The initial composition of the target respondents as indicated by the delegations of the CD-P-PH Committee, steering body, was not completely balanced as regards sectors (see footnote 4).

It is noteworthy that 29.3% of the respondents decided not to disclose the identity of their association/institution, which points to emotional barriers in addressing issues involving the quality of pharmaceutical care.

The reply rate of about 40% to a survey in English and French, official languages of the Council of Europe, sent by e-mail to the respondents is above the usual reply rate for mailed surveys, and indicates great interest in and concern for the quality of pharmaceutical care in Europe throughout the national public health institutions and stakeholder associations. The number and quality of the replies made it possible to draw conclusions on the key questions in the survey.

It is noteworthy that the proportion of patients' associations among the respondents is nearly equal to their representation among the respondents.

Concept

The concept of pharmaceutical care has been endorsed and is promoted by international pharmacists' associations and the World Health Organisation [2]. Literature on the implementation and understanding of the concept of pharmaceutical care in Europe was searched. It should be borne in mind that different terms are used throughout Europe. Recent publications deal mostly with pharmaceutical care in community pharmacy services in several countries in Europe such as Denmark, Estonia Finland, Germany, the Netherlands, the United Kingdom, Portugal, Spain, Sweden, and Switzerland [3-12]. The conclusions of these studies indicate that pharmaceutical care is increasingly being considered an important goal, but is not yet implemented in practice [11,12]. In some cases the basic requirements for the development of pharmaceutical care, such as the availability of patient medication records, still have to be implemented [13]. Studies from countries that are deemed to be relatively active in the field of pharmaceutical care show that implementation in daily practice is still limited, as not all pharmacists provide pharmaceutical care on a routine basis. Only one third of Danish pharmacies report practicing pharmaceutical care [14,15]. Studies from the Netherlands show that the actual number of patients that consult pharmacists in consulting rooms is still very low [16,17]. Although pharmaceutical care activities are expanding in the United Kingdom and the United States, most pharmacists still devote more time to other activities [18,19]. Pharmacists in Northern Ireland reported that they devoted less than 10% of their time to counselling activities [20].

³ Austria, Belgium, Bosnia and Herzegovina: Republika Srpska, Cyprus, Czech Republic, Hungary, Iceland, Ireland, Italy, Latvia, the Netherlands, Norway, Portugal, Switzerland, Turkey, and the United Kingdom

⁴ Respondents: National Public Health Authorities **13.8%** (recipients: 27.2%), national medical doctors' associations **6.9%** (recipients: 15.6%), national pharmacists' associations **29.3%** (recipients: 19.7%), national nurses associations: **6.9%** (recipients: 12.2%), national patients' associations: **12.1%** (recipients: 13.6%), national associations other stakeholders: **1.7%**(11.6%), and "Unknown" **29.3%** (see Chart 1, p. 13, and Chart 2, p. 14)

Patients and carers are widely considered to be essential partners in ensuring the best possible medication outcome, to be agents of managing their condition. All responding pharmacists' associations and public health authorities are of the opinion that pharmaceutical care improves the outcome of patients' medication therapy. These findings are confirmed by literature. The positive effects of pharmaceutical care have been shown by several studies in a wide range of diseases, situations and in different populations: both in the general community, among elderly patients and patients discharged from hospital, and among patients with a wide range of chronic conditions such as diabetes, hypertension, hyperlipidaemia and heart failure and coronary heart disease [21-29]. The majority of respondents, including doctors', nurses' and patients' organisations, rank the importance of the management of medication therapy and the safe use of medicines as high, even if the respondents from doctors', nurses' and patients' organisations did not seem unanimously convinced of the added value of pharmaceutical care. This suggests that the need for pharmaceutical care in the management of medicine therapy is not always recognised by the involved healthcare professionals. Although most respondents agree that pharmaceutical care should involve physicians and pharmacists and, at a different level of involvement, nurses and other healthcare providers, in daily practice in the majority of countries only pharmacists are actively involved. A search of the literature does not readily throw up any studies in which physicians apply the concept of pharmaceutical care. Nurses, incidentally, are reported to be involved in pharmaceutical care [30,31], whereas the concept seems to be less relevant to doctors [32]. On the other hand, the majority of respondents, including those from doctors', nurses' and patients' organisations, rank the importance of the management of medication therapy and the safe use of medicines as high. The term "medication management" is used more frequently by nurses [33]. Although patients do appreciate the provision of care by pharmacists, they still often feel that the physician is the person who primarily is responsible for medication therapy [34,35]. These findings confirm that the concept of pharmaceutical care as a continuous process with the involvement of several healthcare providers is not always appropriately and in a structured manner recognised by all healthcare professionals involved in the medication use chain.

The present survey suggests that the concept of pharmaceutical care as understood by the definition of Helper and Strand (Appendix: survey questionnaire, statement 1) is in general known and accepted in the majority of respondents' countries: the replies indicate that in some cases discussion of the concept has recently started, that interpretation and focus may be different, or that knowledge of the concept is mainly within pharmacists. The degree of implementation of pharmaceutical care in the daily practice of healthcare professionals is limited.

The concept of pharmaceutical care is multi-professional and implies the involvement of the patient: literature on the use and interpretation of the concept of pharmaceutical care by other healthcare professionals such as doctors, nurses and carers, and the contribution and involvement of patients is rarely available, which indicates the need for further research in this field. The implementation of patient care seems to be at different stages in individual countries and requires long-term, systematic and well co-ordinated actions at the local, national and international levels.

It will therefore remain important to co-operate closely with doctors', nurses' and patients' organisations when implementing pharmaceutical care. Considerable effort will be required to make health professionals other than pharmacists acquainted with the concept of pharmaceutical care. Quality indicators could be valuable tools for demonstrating the added value of pharmaceutical care in its multidisciplinary and systemic comprehension.

Terminology

In the majority of countries, the term 'pharmaceutical care' (English) not used frequently, and mainly by pharmacists and in academic circles, in publications and among hospital pharmacists. Some respondents referred to the use of other terms such as 'medication management', "patient counselling", "clinical pharmacy", "pharmaceutical service", "pharmacist care", and "(good) pharmacy practice", describing a specific part of the concept, or a translation of the English term into the local language. Pharmacists and public health authorities more often than other stakeholders are of the opinion that the concept of pharmaceutical care is widely used, which suggests that the concept is not used uniformly in the different sectors. Different sectors and countries put a different focus on the concept and the terminology used.

Comprehensiveness

The majority of respondents considered the concept of pharmaceutical care as proposed by Helper and Strand not comprehensive enough to take account of the roles of all the actors involved in medication therapy. A higher proportion of public health authorities and pharmacists' associations considered the concept to be comprehensive compared to other stakeholders: in particular, doctors and nurses considered the concept to be not sufficiently comprehensive. Respondents who replied that a pharmaceutical care concept which focused on the activities of the pharmacist was not comprehensive, particularly respondents from doctors' and nurses' associations, referred to the lack of a clear definition of the roles of the different healthcare providers and patients.

Barriers

The vast majority of respondents see barriers to the routine provision of pharmaceutical care. Frequent mention was made of a lack of:

- co-operation among healthcare providers;

- education in the field of pharmaceutical care;
- awareness of the concept of pharmaceutical care amongst doctors, pharmacists, nurses and public health authorities.

The fact that patient records are only shared between healthcare providers in a few countries demonstrates the need for improved co-operation among healthcare professionals. Furthermore, reimbursement of pharmacy services is still mostly based on the tasks of pharmacists related to the logistics of the distribution and dispensing of medicines. Similar barriers hindering the routine provision of pharmaceutical care and the expansion of the professional role of the pharmacist were identified in published research [20,36-40]. A recent study carried out in Spain identified barriers to the implementation of pharmaceutical care that included lack of knowledge and the absence of clear and unequivocal messages from professional leaders on the future of pharmacy practice, the lack of reimbursement of cognitive services, the lay-out of pharmacies (e.g lack of privacy), pharmacists' attitudes, the administrative workload, the lack of practical research into pharmaceutical care models, the lack of demand from patients for pharmaceutical care services, and the relationship between pharmacists and doctors [41]. Although some progress has been made in overcoming these barriers, for example through the continuous training of pharmacists throughout Europe, these processes tend to take a long time [42,43].

Implementation of the concept of pharmaceutical care is not given high priority in most countries' policies. Only a few countries have a legal basis for the implementation of pharmaceutical care or have regular contacts between healthcare professionals and insurers in order to implement the concept of pharmaceutical care.

The quality of the management of medicine therapy and the safe use of medicines are not measured on a routine basis in most countries in Europe. Those respondents who refer to measurement refer to the presence of professional standards, drug utilisation data, overall morbidity and mortality statistics and the presence of an adverse drug event reporting system. Although professional standards are important in improving the quality of care, the presence of standards does not imply that medication therapy outcomes are measured. The same holds true for drug utilisation data and other statistics, although the availability of such statistics can be a prerequisite for the development of certain types of indicators [44].

The systems in place at present are primarily focussed on input rather than outcome. Drug regulatory authorities set standards for the quality of medicines. Public health authorities and professional associations set the standards for services, facilities, equipment, and the training of healthcare professionals. These systems are aimed at improving the quality of medication therapy but generally do not measure the clinical outcome for the patient in hospital and ambulatory care environments.

3. RELEVANT ACTIVITIES IN THE FIELD OF THE MEASUREMENT OF THE QUALITY OF HEALTHCARE IN EUROPE - WHAT IS THE ADDED VALUE OF THE DEVELOPMENT OF INDICATORS FOR PHARMACEUTICAL CARE?

Several other initiatives for developing quality indicators in healthcare are currently under way at the national level in Europe and as the result of international co-operation. This overview is limited to the latter. The European Commission (EC) published a strategy on European Community Health Indicators (ECHI) and the Organisation for Economic Co-operation and Development (OECD) developed a set of Health Care Quality Indicators (HCQI). In the table below the differences and similarities are presented on several relevant aspects of indicator development.

Table 1

	OECD	EC DG Sanco	CoE/EDQM
Name	HCQI	ECHI(M) ISARE Sogeti report PPRI project	CD-P-PH "basic indicators" in pharmaceutical care
Year(s)	2002 - ongoing	2004 - ongoing	2008 - 2010
Reach	30 countries	300 defined regions (ISARE) and 'wider EU' (others)	37 CoE countries
Vision / goal	Improve prosperity by improving competitiveness and performance of pharmaceutical industry, with a view on public health requirements Use open standard performance benchmarks	Develop comparable information and knowledge on health related behaviour, diseases and health systems, fully accessible to experts and the public (public health programme)	Facilitate development of standards on quality and safety of use of medication. Provide guidance and tools (indicators) to assess the quality of pharmaceutical care and medication safety

Priority areas	Mental health Patient safety Health promotion, prevention and primary care Patient experiences/ responsiveness	Priority diseases (WHO-based) Disease status Availability of medicines Affordability of medicines Market accessibility of medicines Effectiveness of medicines Innovation	Definitions and common language Indicators methods Medication Safety Patients' involvement and concordance Communication and co-operation in pharmaceutical care
Typical subjects (not limitative)	Myocardial infarction Chronic heart failure Stroke Hip fracture Cancer Asthma Diabetes Communicable diseases	Death rate DALY Availability of therapies Exposure to risk factors Death and adverse events caused by medicines Cost-effectiveness (ICER) R&D expenditure in terms of added value Medicines in development Patents granted Market share of generics Time between first application and launch Uptake of new medicines	
Areas of expert consultation	• Quality of data sources • Standardisation by age and sex • Comparability • Primary care • Patient safety • Responsiveness	• Lifestyles • Morbidity and mortality • Health systems • Health and environment • Mental health • Accidents and injuries • Community health indicators	• Pharmaceutical care • Medication safety • Methodology • Multidisciplinarity
Measurement cycle	2 years	1 - 5 years depending on indicator	To be established
Number of indicators	Regular: 15 New: 7 or more	10 core indicators and 10 supplementary indicators ready 25 potential indicators require further work	Long list, aimed at producing a first basic set of 5 to 10 pilot indicators

As shown in Table 1, the scope and purpose of the individual projects differ from the project and approach CD-P-PH/PC Committee of Experts project and approach which is focused on the safe use of medicines and concordance. However, it is important that each of the identified initiatives share progress and discuss common issues as the projects proceed.

4. CONCLUSIONS AND RECOMMENDATIONS

Based on the survey findings, including the input from the P-SP-PH/SAFE former Expert Group, scientific publications, and a consultation with the CD-P-PH/PC Committee of Experts Quality of Pharmaceutical Careworking party, the following conclusions and recommendations have been drawn up.

Conclusions

Concept and definition of pharmaceutical care

Although the concept of pharmaceutical care as understood by the definition of Helper and Strand (Appendix: survey questionnaire, statement 1) is known and not objected to in the majority of respondents' countries, the concept is not yet widely used in the majority of countries. There is a lack of common understanding on the principles of pharmaceutical care, its objectives and practical requirements. There are historical and cultural reasons for the differences that exist in the interpretation of pharmaceutical care between the different countries and professions, and will remain valid in the future.

Use of the concept of pharmaceutical care

It is broadly recognised that there is a need for multidisciplinary collaboration between those involved in the medication use chain in order to ensure the safe and effective use of medicines. Furthermore, the implementation of pharmaceutical care should be advanced. Currently, it is mainly pharmacists who are involved in pharmaceutical care activities. The provision of pharmaceutical care requires certain competences and skills which are not exclusive to the current qualifications of pharmacists, but which connect well. There is little guidance on quality across the professions involved in the medication use chain.

Quality of pharmaceutical care indicators

Although there is a consensus that improving and ensuring the safety and quality of the medication use chain should be a priority, there is no common approach and strategy in Europe on how to develop an assessment and monitoring tool such as quality indicators. Only a small minority of countries in Europe have developed indicators to either measure the quality of care or the satisfaction of patients with the delivery of healthcare.

There is a substantial lack of suitable measures of outcomes. Measures of outcome are currently considered useful on local and national levels for quality improvement and from the perspective of public health monitoring. Quality indicators may become suitable measures of outcome in the medication use chain. In cases where pharmaceutical care cannot clearly be linked to clinical outcomes, process indicators may still be adequate.

It is important that each of the initiatives dealing with pharmaceutical care or related issues at the national and international levels in Europe share reporting on progress and discuss common issues as the projects proceed.

Databases

Some countries report the presence of drug utilisation data and adverse drug event reporting databases. However, these data are not directly useful as quality indicators at the European level. Some of these databases might prove useful for specific indicators at the national level.

Patient involvement

Concordance between healthcare professionals and patients and joint decision-making are at the heart of any care process, including pharmaceutical care. The concept of concordance has not yet been adopted throughout Europe.

Political and managerial context

Authorities make vast efforts to maintain a stringent system of licensing of medicines, whereas relatively little effort is made to ensure the best use of these high-quality medicines in daily practice.

Recommendations

Concept and definition of pharmaceutical care

In order to implement pharmaceutical care as a successful strategy for improving outcomes of medication therapy, public understanding needs to be fostered. The definition of pharmaceutical care should take account of Helper and Strand's definition of pharmaceutical care⁵.

⁵ Pharmaceutical care is the responsible provision of drug therapy for the purpose of achieving definite outcomes that improve a patient's quality of life. These outcomes are:

- cure of a disease;
- elimination or reduction of a patients' symptomatology;
- arresting or slowing of a disease process; or
- preventing a disease or symptomatology.

Pharmaceutical care involves the process that a pharmacist co-operates with the patient and healthcare professionals in designing, implementing, and monitoring a therapeutic plan that will produce specific therapeutic outcomes for the patient. This in turn involves three major functions:

- identifying potential and actual drug-related problems;
- resolving actual drug-related problems; and
- preventing drug-related problems.

Pharmaceutical care is a necessary element of healthcare, and should be integrated with other elements. Pharmaceutical care is, however, provided for the direct benefit to the patient, and the pharmacist is responsible directly to the patient for the quality of that care. The fundamental relationship in pharmaceutical care is a mutually beneficial exchange in which the patient grants authority to the provider and the provider gives competence and commitment (accept responsibility) to the patient. These fundamental goals, processes, and relationships of pharmaceutical care exist regardless of practice setting and of professional background.

It is recommended that the term “pharmaceutical care” be used in preference to other terms such as “medication management” and “medicine management”, as it emphasises the care-providing aspect in addition to the ‘classical’ roles of pharmacy in terms of product quality and medication logistics.

Use of the concept and definition of pharmaceutical care

International guidance documents on the quality of pharmaceutical care should also include the perspectives of pharmacists and all healthcare professionals involved in the medication use chain. In order to foster the implementation of pharmaceutical care among pharmacists and other professionals, the added value of specific pharmaceutical care activities needs to be clearly demonstrated.

A set of principles must be established on how to share medical and medication-related information between all the players in the medication use chain.

Effective collaboration demands the sharing of information and health records. Co-operation structures are needed to support the multidisciplinary character of pharmaceutical care activities. Examples are standards for electronic information exchange, common syntax and coding, safe networking facilities, and unique patient and care-provider identification.

Good pharmaceutical care practices need to be developed.

Quality of pharmaceutical care indicators

In developing indicators for pharmaceutical care, it is important to select clearly defined areas and activities that are acceptable in a wide range of European countries.

It is recommended that a comprehensive list be drawn up of those areas where indicators would be desirable and those where indicators already exist. From this list a limited set of basic indicators could be selected for further development and testing. It is important that these basic indicators can be applied throughout Europe. Co-operation between countries with different histories of pharmaceutical care, medical traditions and healthcare systems should be strongly encouraged.

Databases

It is recommended that the usefulness of international databases be investigated, for instance at WHO, OECD or EU level, once specific quality indicator subjects have been selected.

Consideration should be given to supplementing existing medical coding standards with extensions for pharmaceutical care.

A strict harmonisation of underlying data collection methods is essential in order to obtain quality indicators that are comparable between countries.

Patient involvement

Patient participation and commitment must be improved in order to contribute to good outcomes of pharmaceutical care. Patient expectations and desired quality of life are important factors to ensure the best possible medication outcome, and to prevent recurrence of disease, as feasible. An indicator for patient participation should be developed.

Medication-related health literacy should be promoted within the general public.

Communication – getting everyone on board

The concept of pharmaceutical care and related activities needs to be promoted among all the stakeholders, including the public health authorities, doctors, pharmacists, nurses, patients and carers. All the stakeholders need to be involved in an appropriate way; this is an essential element for successful promotion of the concept in order to achieve improved outcomes for patients. Basic communication and information programmes need to be undertaken systematically and regularly.

Political and managerial context

National healthcare authorities could provide a legal basis for removing the various barriers to the implementation of pharmaceutical care that exist currently.

Outcome measurements are needed in order to assess the possible effects achieved during the care-giving process, including pharmaceutical care. The indicator-based approach is expected to be effective in improving safety and quality. Further gathering experience of and evidence for using indicators as a tool for the evaluation of healthcare policies through systematic research at the national and European levels would be desirable.

A general policy should be developed in order to involve the various stakeholders in pharmaceutical care as appropriate for implementation of the concept.

Healthcare systems need to take account of pharmaceutical care as a working method for the promotion of the safety and quality of medication use. This may be achieved in a variety of ways and should be assessed and monitored, using specific indicators, for example.

Minimum training requirements should be established across the professions, using both under- and postgraduate university education and training.

The authors of the study are convinced that it is necessary to develop indicators of the quality of patients' clinical medication outcome and to investigate their impact on the outcomes of medication in patients throughout Europe.

The preliminary survey results indicate that the multi-professional concept of pharmaceutical care is generally known throughout Europe and not objected to. There is a need for advice and support for implementation, in particular in the community environment. Political support and national health policies in this field are necessary; the authors of the study are of the view that the survey findings suggest that the promotion of pharmaceutical care in Europe is a comprehensive undertaking with a wide and far-reaching impact on patients well-being and on healthcare services.

The CD-P-PH committee has been entrusted, through its mandate and competences in patient-oriented public health policies, with improving medication outcomes for patients in Europe by the promotion of pharmaceutical care by means of specific policies and measures.

As a first step, it is recommended that a basic set of quality indicators should be developed with the assistance of several academic and public health institutions within the framework of the programme of activities steered by the CD-P-PH committee in order to help monitor pharmaceutical care in Europe. This preparatory work could give rise to both the development of a platform to help with testing, implementing and evaluating the quality indicators across Europe, and the appropriate use of the information obtained for the establishment of specific public health oriented policies.

The platform could also assist in the pooling experiences, best practices and relevant know-how related to pharmaceutical care across Europe and promotion of the political will to foster the quality of pharmaceutical care as a comprehensive concept, focused on the patient's quality of life and involving all the appropriate stakeholders.

5. METHODS

In co-operation with the responsible EDQM Administrator, who provided scientific and technical assistance, the CD-P-PH/PC Committee of Experts "Quality of Pharmaceutical Care" working party prepared the draft survey questionnaire "The definitions of key concepts in pharmaceutical care, indicators for good quality of pharmaceutical care and services, existing sources for relevant data/information in the states Parties to the Convention on the Elaboration of a European Pharmacopoeia (Ph. Eur.), observers to the Ph. Eur., and other Council of Europe member states".

In line with its decisions taken at its 61st meeting on 19-21 May 2008, the CD-P-PH/PC Committee of Experts approved the draft survey questionnaire in writing on 16 October 2008. The survey questionnaire was submitted to the national key stakeholders in pharmaceutical care indicated by the delegations of CD-P-PH Committee, steering body, in reply to the request for indicating the contact details of national stakeholders in pharmaceutical care. As requested by the delegations of the CD-P-PH the survey questionnaire was sent either via the Heads of the Liaison sections of the Partial Agreement of the European Pharmacopoeia or directly by the Secretariat. Nineteen (19) CD-P-PH delegations from 11 EU and 8 non-EU states parties of the above mentioned Ph. Eur. convention sent contact details from national (public) health authorities, doctors', pharmacists', nurses' and patients' associations (see chapter 6, Table 2, Chart 1).

The survey questionnaire was sent to the recipients with a deadline for reply by 20 December 2009.

The survey software "Survey Monkey[®]" was used by the Secretariat to carry out the online-survey and to collect the replies. The survey questionnaire was bilingual in English and French, the two official languages of the Council of Europe. The survey comprised background information on Helper and Strand's definition of pharmaceutical care by and other background information necessary for answering the the survey questionnaire (see Appendix). Depending on the requirements of the national CD-P-PH delegations, the target respondents were provided by e-mail with the link to the survey or included in the survey software's distribution list and sent an e-mail with the link via the survey software.

The e-mail message contained administrative information such as the basic information about the EDQM and the bodies responsible for the survey, survey objectives, possible follow-up to the survey and its report, technical information such as how to process and complete the survey, and a confidentiality statement for individual replies and the identity of individual respondents. The Secretariat followed up the initial e-mail by phoning all target respondents to make sure that they had received the e-mail and to ask if they had any technical problems with the online-survey.

By 16 December 2008 all recipients were contacted by e-mail reminding them to either check that they had indicated their contact details (recipients co-ordinated by the Liaison sections) or had sent a reply. The survey data were tabulated using MS Office software.

Preparation of the study report

In cooperation with the members of the CD-P-PH/PC Committee of Experts "Quality of Pharmaceutical Care" working party, the consultant prepared a report comprising a narrative summary and a discussion of key trends in the information obtained by the survey, key recommendations for the development of indicators for the quality of

pharmaceutical care in ambulatory and clinical environments from a multi-professional perspective, and a detailed discussion of the compiled individual survey replies. The consultant looked on the data from the perspective of the development and application of indicators for the quality of pharmaceutical care in ambulatory and clinical environments from a multi-professional perspective, including patients' participation. The discussion took into account accepted current scientific knowledge in the field of indicators for the quality of pharmaceutical care.

The report was prepared in two stages between 15 January and the end of March 2009.

6. RECIPIENTS AND RESPONDENTS

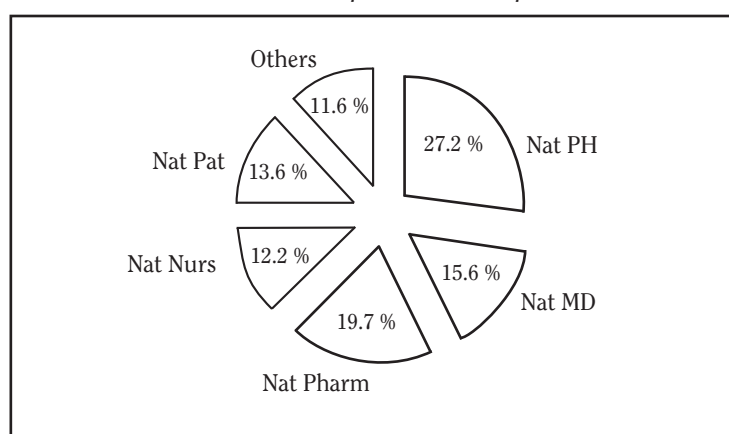
The survey questionnaire was sent to 147 recipient institutions/associations (national public health authorities, national doctors', national pharmacists', national nurses', others [e.g. associations for disabled persons], and patients' associations) (see Table 2, Chart 1). In addition, the questionnaire was sent to 6 members, doctors, pharmacists, and a nurse, from the former P-SP-PH/SAFE Group of Experts from France, Georgia, Germany, and Sweden, countries not represented in the recipients' countries (see Chapter 2).

To ensure the confidentiality of individual replies and their origin, the Secretariat assigned random code numbers to the countries. The recipients were divided into the sectors of national public health authorities ("nat PH"), national doctors' ("nat MD"), pharmacists' ("nat Pharm"), nurses' ("nat Nurs"), "others" (e.g. associations for disabled persons), and patients' associations ("nat Pat").

Table 2: Recipients

Total	Nat PH	Nat MD	Nat Pharm	Nat Nurs	Nat Pat	Others
147	40 (27.2%)	23 (15.6%)	29 (19.7%)	18 (12.2%)	20 (13.6%)	17 (11.6%)

Chart 1: Composition of recipients



Out of the 78 replies received from national public health authorities, national doctors', pharmacists', nurses', others (associations for disabled persons) and patients' associations from 17 out of 19 countries^{2,3} (see Table 3, Chart 2), 58 replies could be evaluated. For those respondents who decided not to disclose their contact details, the sector "Not known" was introduced.

Among the remaining 20 replies, 8 replies did not contain concrete information, but had apparently been used to 'screen' the obligatory questions of the questionnaire, and 12 replies from the same Internet Protocol (IP) address were started and finalised at different time points. The most recent reply was considered for evaluation.

Six (6) complete replies were also received from 6 experts from the former P-SP-PH/SAFE Group of Experts. The information supplied by the experts was evaluated and presented separately.

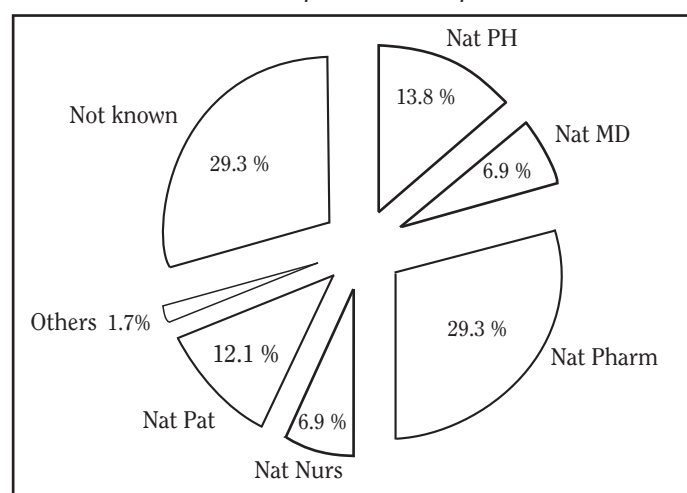
Table 3: Number of respondents per country and sector

COUNTRY N=17	Code	Nat PH	Nat MD	Nat Pharm	Nat Nurs	Nat Pat	Others	Unknown	Respondents/ country
1	11	1		2					3
2	13	1		2				2	5
3	18			1				1	2
4	19				3	1		2	6
5	22	1	1					1	3
6	26			1				2	3
7	33			1				1	2
8	38	1		1	1			1	4
9	39			1					1
10	47	1		1		1			3
11	54	2				2		4	8
12	56		1	1		2		1	5
13	63			1			1		2
14	77			1					1
15	85		1						1
16	92	1	1	2					4
17	93			2		1		2	5
Total		8 (13.8%)	4 (6.9%)	17 (29.3%)	4 (6.9%)	7 (12.1%)	1 (1.7%)	17 (29.3%)	58 (100%)

The representation of the different sectors in the respondents was as follows:

“Nat PH”: **13.8%** (recipients: 27.2%), “nat MD”: **6.9%** (recipients: 15.6%), “nat Pharm”: **29.3%** (recipients: 19.7%), “nat Nurs”: **6.9%** (recipients: 12.2%), “nat Pat”: **12.1%** (recipients: 13.6%), “nat Others”: **1.7%**(11.6%), and “not known” **29.3%**.

Chart 2: Composition of respondents



Although the replies from pharmacists' associations were the most numerous, the reply rates of national public health authorities, patients' associations and other healthcare professionals' associations (e.g. doctors and nurses) were sufficient in number and quality to be able to draw conclusions on the fundamental questions of the survey.

It should be borne in mind that the composition of the target respondents as indicated by the delegations of the CD-P-PH Committee of Experts were not balanced in terms of sector. It is noteworthy that 29.3% of the respondents decided not to disclose the identity of their association/institution, which points to emotional barriers in addressing issues concerning the quality of pharmaceutical care.

7. REPLY RATES

According to relevant literature, the standard reply rate for surveys returned by post or e-mail is about 10-20%. The reply rate of about 40% for this survey is above the expected reply rate. The survey was sent out in English and

French, official languages of the Council of Europe, to the recipients, a majority of whom non-native speakers of both languages. Considering different levels of mastering English or French by national patients' associations and healthcare professionals across the respondents' countries, the reply rate demonstrates great interest in and concern across the national public health institutions and stakeholder associations for the quality of pharmaceutical care in Europe. It is noteworthy that the proportion of patients' associations in the respondents is nearly equal to their representation among the recipients.

The replies were either complete or partially completed (see Table 4).

Table 4: Individual questions – proportion of replies

Question No	Replies/question	Replies (%)
Total respondents	58	
1	57	98.3
2	57	98.3
3	57	98.3
4	55	94.8
5	55	94.8
6	38	65.5
7	43	74.1
8	46	79.3
9	42	72.4
10	46	79.3
11	46	79.3
12	45	77.6
13	45	77.6
14	43	74.1
15	44	75.9
16	44	75.9
17	37	63.8
18	43	74.1
19	41	70.7
20	43	74.1
21	41	70.7
22	39	67.2
23	41	70.7
24	37	63.8
25	37	63.8
26	38	65.5
27	37	63.8
28	36	62.1
29	35	60.3
30	35	60.3
31	34	58.6
32	37	63.8
33	33	56.9
35*	14	24.1

* Q 35: General comments on the survey questionnaire - voluntary

8. DETAILED DISCUSSION OF SURVEY RESULTS

Note: Six (6) experts** (column “Experts”) from the former P-SP-PH/SAFE Group of Experts were invited to send replies. The information supplied by the experts was evaluated and presented separately and not included in the overall evaluation (column “Total”).

** France, Germany, Sweden, and Georgia

I. Understanding of the concept “Pharmaceutical Care”

Q.1 Do you think the concept of “pharmaceutical care”, see statement 1, is known in your country?*

	Sectors	n	PH	n	MD	n	Pat	n	Nurs	n	Others	n	Unknown	n	Total	Experts
	Pharm	10	87.5%	7	50.0%	2	71.4%	5	50.0%	2			82.4%	14	% n=58	%
Yes	58.8%	10	87.5%	7	50.0%	2	71.4%	5	50.0%	2			82.4%	14	69.0%	66.7% 4
Partly	35.3%	6	12.5%	1	25.0%	1	14.3%	1	50.0%	2			0.0%		19.0%	16.7% 1
No	5.9%	1	0.0%		0.0%		14.3%	1	0.0%		100%	1	11.8%	2	8.6%	16.7% 1
Do not know	0.0%		0.0%		0.0%		0.0%		0.0%				5.9%	1	1.7%	0.0%
Missing	0.0%		0.0%		25.0%	1	0.0%		0.0%				0.0%		1.7%	0.0%
	100%	17	100%	8	100%	4	100%	7	100%	4	100%	1	100%	17	100%	100% 6

Conclusions

The concept of pharmaceutical care as understood in statement 1 (Helper and Strand definition; see Appendix: survey questionnaire) is known at least in parts by the majority of respondents. 'Partly' means that some respondents stated that the concept is only known among pharmacists or that discussion of the concept is only recent. There seems to be no major difference between sectors, although the fact that the overall higher response from public health institutions and pharmacists suggests that these sectors are more familiar with the concept. The experts' replies were in line with the respondents' views.

* See Appendix, survey questionnaire

Q.2 Do you think the term “pharmaceutical care”^{***}, see statement 1*, is widely used in your country?

	Sectors	n	PH	n	MD	n	Pat	n	Nurs	n	Others	n	Unknown	n	Total	n=58	Experts	n=6
Yes	Pharm	52.9%	9	37.5%	3	25.0%	1	42.9%	3	0.0%			70.6%	12	48.3%	28	16.7%	1
Partly		29.4%	5	50.0%	4	0.0%		14.3%	1	50.0%	2		5.9%	1	22.4%	13		
No		17.6%	3	12.5%	1	50.0%	2	42.9%	3	50.0%	2	100%	23.5%	4	27.6%	16	83.3%	5
Missing		0.0%		0.0%		25.0%	1	0.0%		0.0%			0.0%		1.7%	1		
		100.0%	17	100%	8	100%	4	100%	7	100%	4	100%	100%	17	100.0%	58	100%	6

Conclusions

Approximately half the respondents stated that the term “pharmaceutical care” is widely used. Some respondents specifically stated that use of the concept seemed to be favoured only by pharmacists and sometimes only in academic circles, for publications, or only among hospital pharmacists. Some countries stated that another term was used to describe the concept or specific aspects. Pharmacists and public health authorities more often than other stakeholders are of the opinion that the concept of pharmaceutical care is widely used, which suggests that the term is not used uniformly across the different sectors. The experts’ replies were in line with the respondents’ views.

* See Appendix, survey questionnaire

Q.3 The concept of pharmaceutical care, statement 1*, focuses on the activities of the pharmacist. Do you think the concept is sufficiently comprehensive to take account of the roles of all stakeholders of medicine therapy?

	Sectors	n	PH	n	MD	n	Pat	n	Nurs	n	Others	n	Unknown	n	Total	Experts	
	Pharm															n=58	%
Yes	70.6%	12	75.0%	6	0.0%		42.9%	3	25.0%	1			58.8%	10	55.2%	32	0.67%
Partly	5.9%	1	12.5%	1	0.0%		0.0%		0.0%				0.0%		3.4%	2	0.00%
No	23.5%	4	12.5%	1	75.0%	3	42.9%	3	75.0%	3	100%	1	41.2%	7	37.9%	22	0.33%
Unknown	0.0%		0.0%		0.0%		14.3%	1	0.0%				0.0%		1.7%	1	0.00%
Missing	0.0%		0.0%		25.0%	1	0.0%		0.0%				0.0%		1.7%	1	0.00%
	100.0%	17	100%	8	100%	4	100%	7	100%	4	100%	1	100%	17	100%	58	1.00%
																	n=6

Conclusions

Whereas half of the respondents agree that the concept is sufficiently comprehensive, more than a third of respondents disagree. Public health authorities and pharmacists' associations rate the comprehensiveness of the concept higher than other stakeholders. Respondents who replied that a pharmaceutical care concept which focused on the activities of the pharmacist was not comprehensive referred more particularly to the lack of a clear definition of the roles of the different healthcare providers and patients in providing pharmaceutical care. The experts' replies were in line with the respondents' views.

* See Appendix, survey questionnaire

Q.4 Which alternative terms are used for the concept of “pharmaceutical care” in your country?

	Sectors	n	PH	n	MD	n	Pat	n	Nurs	n	Others	n	Unknown	n	Total	Experts	n=58	%	n=6
Term is translated into language of respondent's country	Pharm	3	12.5%	1	0.0%		14.3%	1	0.0%				5.9%	1	10.3%		6	0.0%	
Same term	41.2%	7	37.5%	3	50.0%	2	0.0%		25.0%	1			41.2%	7	34.5%		20	0.0%	
Alternative term, see below	29.4%	5	50.0%	4	25.0%	1	14.3%	1	75.0%	3			41.2%	7	36.2%		21	66.7%	4
Do not know	11.8%	2	0.0%		0.0%		14.3%	1	0.0%				5.9%	1	6.9%		4	16.7%	1
No relevant answer	0.0%		0.0%		0.0%		57.1%	4	0.0%				0.0%		6.9%		4	0.0%	
Missing	0.0%		0.0%		25.0%	1	0.0%		0.0%		100%	1	5.9%	1	5.2%		3	16.7%	1
	100.0%	17	100%	8	100%	4	100%	7	100%	4	100%	1	100%	17	100%		58	100%	6

Alternative terms

In several countries the term “pharmaceutical care” was translated into the local language. Only 4 countries have no alternative term for pharmaceutical care. Respondents from public health authorities and nurses’ associations more particularly referred to other terms. Respondents from 13 countries reported the use of a wide range of terms such as ‘medicines management’, ‘(contemporary) pharmaceutical services’, ‘pharmacist care’, ‘good pharmacy practice’, ‘pharmaceutical awareness’, ‘pharmaceutical therapy/treatment’, ‘patient counselling’, ‘good communication with the patient’ and ‘clinical pharmacy’. Several of these terms are used in international literature. Some of these alternative terms describe activities that are part of pharmaceutical care but do not cover the whole area of pharmaceutical care. This adds to the idea that the concept of pharmaceutical care is not yet recognised in its entirety in all countries in Europe.

The majority of the respondents are of the opinion that in the activities mentioned in statement 1 pharmacists, doctors nurses and other healthcare professionals should be involved. Most respondents also mention other healthcare professionals, such as dieticians, patients' carers, midwives, physiotherapists, and dentists which should be involved as well. This is in line with the concept of pharmaceutical care that applies to all healthcare professionals.

*See Appendix, survey questionnaire

	Pharm	n	PH	n	MD	n	Pat	n	Nurs	n	Others	n	Unknown	n	Total	Experts
Only pharmacists	11.8%	2	25.0%	2	0.0%		14.3%	1	0.0%				5.9%	1	10.3%	n=58 %
Only doctors	0.0%		0.0%		0.0%		0.0%		0.0%				11.8%	2	3.4%	6 0.0%
Pharmacists and doctors	11.8%	2	12.5%	1	25.0%	1	42.9%	3	0.0%				0.0%		12.1%	2 0.0%
Pharmacists, doctors and nurses	29.4%	5	25.0%	2	50.0%	2	0.0%		25.0%	1			17.6%	3	22.4%	7 50.0%
Pharmacists, doctors nurses and other healthcare professionals	41.2%	7	37.5%	3	0.0%		28.6%	2	75.0%	3			41.2%	7	37.9%	22 3
Only health care politicians and some physicians	0.0%		0.0%		0.0%		0.0%		0.0%				5.9%	1	1.7%	1 0.0%
Only doctors and nurses	0.0%		0.0%		0.0%		0.0%		0.0%				5.9%	1	1.7%	1 0.0%
No relevant answer	5.9%	1	0.0%		0.0%		0.0%		0.0%				11.8%	2	5.2%	3 0.0%
Missing	0.0%		0.0%		25.0%	1	14.3%	1	0.0%		100%	1	0.0%		5.2%	3 0.0%
	100.0%	17	100%	8	100%	4	100%	7	100%	4	100%	1	100%	17	100%	58 100%

II. Use of the concept “Pharmaceutical care”

Q.6 To what extent is the concept of pharmaceutical care, see statement 1*, applied practically by representatives of the following professions? If you provide information for other stakeholder(s), please specify them here: e.g. physiotherapists, etc.

To what extent is the concept of pharmaceutical care applied practically by pharmacists?

	Sectors														Total		Experts	
	Pharm	n	PH	n	MD	n	Pat	n	Nurs	n	Others	n		%	n=58	%	n=6	
Concept not applied practically	0.0%		0.0%		0.0%		28.6%	2	25.0%	1				8.6%	5	0.0%		
Concept considered desirable, few apply	23.5%	4	37.5%	3	50.0%	2	14.3%	1	25.0%	1				25.9%	15	33.3%	2	
Concept common practice	41.2%	7	25.0%	2	0.0%		28.6%	2	25.0%	1				24.1%	14	33.3%	2	
Concept minimum standard, to some extent	0.0%		25.0%	2	25.0%	1	0.0%		0.0%					8.6%	5	0.0%		
Missing	35.3%	6	12.5%	1	25.0%	1	28.6%	2	25.0%	1	100%	1		32.8%	19	33.3%	2	
	100.0%	17	100%	8	100%	4	100%	7	100%	4	100%	1		100%	58	100%	6	

To what extent is the concept of pharmaceutical care applied practically by doctors?

	Sectors	n	PH	n	MD	n	Pat	n	Nurs	n	Others	n	Total	%	n=58	Experts	%	n=6
Concept not applied practically	Pharm 23.5%	4	12.5%	1	25.0%	1	28.6%	2	25.0%	1				17.2%	10	16.7%		1
Concept considered desirable, few apply	23.5%	4	25.0%	2	0.0%		28.6%	2	0.0%				19.0%		11	50.0%		3
Concept common practice	5.9%	1	25.0%	2	25.0%	1	0.0%		25.0%	1			10.3%		6	16.7%		1
Concept minimum standard, to some extent	5.9%	1	0.0%		0.0%		28.6%	2	25.0%	1			6.9%		4	0.0%		
Missing	41.2%	7	37.5%	3	50.0%	2	14.3%	1	25.0%	1	100%	1	46.6%		27	16.7%		1
	100.0%	17	100%	8	100%	4	100%	7	100%	4	100%	1	100%		58	100%		6

* See Appendix, survey questionnaire

To what extent is the concept of pharmaceutical care applied practically by nurses?

	Sectors	n	PH	n	MD	n	Pat	n	Nurs	n	Others	n	Total	Experts
	Pharm	3	12.5%	1	25.0%	1	0.0%		0.0%				%	n=58
Concept not applied practically	17.6%	3	12.5%	1	25.0%	1	0.0%		0.0%				13.8%	8 66.7% 4
Concept considered desirable, few apply	23.5%	4	0.0%		25.0%	1	28.6%	2	25.0%	1			15.5%	9 0.0%
Concept common practice	5.9%	1	12.5%	1	25.0%	1	14.3%	1	25.0%	1			12.1%	7 0.0%
Concept minimum standard, to some extent	0.0%		0.0%		0.0%		0.0%		25.0%	1			1.7%	1 16.7% 1
Missing	52.9%	9	75.0%	6	25.0%	1	57.1%	4	25.0%	1	100%	1	56.9%	33 16.7% 1
	100.0%	17	100%	8	100%	4	100%	7	100%	4	100%	1	100%	58 100% 6

To what extent is the concept of pharmaceutical care applied practically by other healthcare professionals?

[illegible]

Conclusions

The results suggest that the various stakeholders consider pharmacists to be particularly involved in pharmaceutical care. But even pharmacists think that pharmacists do not apply pharmaceutical care always in daily practice. The experts' replies were in line with the respondents' views. Some respondents' explanations describe the current status of the application of pharmaceutical care

- 'There is no obligation to apply the concept of pharmaceutical care, but sub-domains of it are being applied since long: e.g. check of prescribed doses, identification, prevention and resolving drug-related problems. Community pharmacists will have to apply the full concept in a few years time.'
- 'Pharmaceutical care is not provided on a formal structured basis in our country. Therefore there is no consistency for patients as to the level of care, its standard and quality, or as yet consistency in the specified standards by which practitioners may measure their performance. This is also related to the fact that there is a lack of uniform interpretation of the concepts.'
- 'Communication between the different healthcare professionals (in particular pharmacists, doctors and nurses) needs to be improved and a more planned and shared approach taken. Patients need better information and need to be encouraged to discuss issues.'

Q.7 How would you describe the role in pharmaceutical care of: pharmacists, doctors, nurses, other healthcare professionals, patients, carers?

Pharmacists

	Sectors	n	PH	n	MD	n	Pat	n	Nurs	n	Others	n	Total	Experts
	Pharm	9	50.0%	4	50.0%	2	28.6%	2	50.0%	2		Unknown n=58	% n=58	n=6
Key/important role	52.9%	9	50.0%	4	50.0%	2	28.6%	2	50.0%	2		23.5% 4	39.7% 23	16.7% 1
Supportive to doctors	0.0%		0.0%		0.0%		0.0%		50.0%	2		0.0% 	3.4% 2	0.0%
Specific role: see conclusion	29.4%	5	37.5%	3	50.0%	2	57.1%	4	0.0%			11.8% 2	27.6% 16	83.3% 5
Negligible	0.0%		0.0%		0.0%		0.0%		0.0%			5.9% 1	1.7% 1	0.0%
Missing	17.6%	3	12.5%	1	0.0%		14.3%	1	0.0%		100%	1 58.8% 10	27.6% 16	0.0%
	100.0%	17	100%	8	100%	4	100%	7	100%	4	100%	1 100% 17	100% 58	100% 6

Conclusions

Most respondents, especially pharmacists themselves, are of the opinion that the pharmacist has a key (important, central, pivotal, crucial, maximum) role to play in pharmaceutical care. Nurses in particular reported that the role of the pharmacist is to back up the doctor's work. Several respondents tried to describe the roles of the pharmacist in more detail. Some examples of the specific role of the pharmacist as described as follows:

- ‘.... to be beyond accurately dispensing prescriptions to ensure that prescribed medicines are safe and effective for the individual patient; checking the prescribed drug’s applicability related to the indication, its safety and possible interactions with other prescribed drugs, advice in health and nutrition and prior contact point for a lot of health issues.’
- ‘Informing and advising patients about their medicines; document the patient’s medical condition(s); document all medication; review of medication (interactions, double medication, dosage) and medication use (adherence, observed side-effects); participating in continuing education programmes; participating in regular local multi-disciplinary team meetings.’
- ‘Good communication with the patients; good collaboration with the other health team members. Consulting both (patients, other health professionals) on pharmacotherapy, pharmacological aspects. For patients: prevention of diseases; monitor chronic conditions in patients.’

Doctors

	Sectors	n	PH	n	MD	n	Pat	n	Nurs	n	Others	n	Unknown	n	Total	n=58	Experts
	Pharm	3	0.0%		25.0%	1	28.6%	2	50.0%	2			11.8%	2	17.2%	10	0.0%
Key/important role	17.6%	3	0.0%		25.0%	1	28.6%	2	50.0%	2			11.8%	2	17.2%	10	0.0%
Cooperative, together in a team	52.9%	9	50.0%	4	0.0%		0.0%		0.0%				11.8%	2	25.9%	15	16.7%
Specific role: see conclusion	0.0%		25.0%	2	75.0%	3	42.9%	3	50.0%	2			11.8%	2	20.7%	12	66.7%
None	0.0%		0.0%		0.0%		0.0%		0.0%				5.9%	1	1.7%	1	0.0%
Missing	29.4%	5	25.0%	2	0.0%		28.6%	2	0.0%		100%	1	58.8%	10	34.5%	20	16.7%
	100.0%	17	100%	8	100%	4	100%	7	100%	4	100%	1	100%	17	100%	58	100%
																	n=6

Conclusions

All stakeholders, including doctors themselves, rate the role of doctors in pharmaceutical care as being less frequently of key importance than that of pharmacists. However, respondents from all stakeholders recognise the importance of teamwork and co-operation. The specific roles of the doctor as mentioned by some respondents are:

- ‘Should be a strong primary focus of care, primary responsibility for prescribing effective and safe treatment in the right dosage, diagnosis, preventing and reduction of diseases, improving rational drug use, rational prescribing of medicines.’
- ‘As the prescriber, the doctor should play a leading role with the pharmacist in the development, delivery and monitoring of therapeutic care plans.’
- ‘Ensure compatibility of all the patient’s medication, obtain patient feedback, and adapt medical therapy, pharmacovigilance.’
- ‘Co-operation with pharmacists at analysis and solution of possible drug-related problems.’

Nurses

	Sectors	n	PH	n	MD	n	Pat	n	Nurs	n	Others	n	Unknown	n	Total	Experts
	Pharm														n=58	%
Key/important role	11.8%	2	0.0%		25.0%	1	0.0%		50.0%	2			5.9%	1	10.3%	6
Cooperative, together in a team	41.2%	7	25.0%	2	0.0%		14.3%	1	0.0%				0.0%		17.2%	10
Specific role: see conclusion	17.6%	3	50.0%	4	50.0%	2	42.9%	3	50.0%	2			11.8%	2	27.6%	16
Passive/minimal	0.0%		0.0%		0.0%		14.3%	1	0.0%				11.8%	2	5.2%	3
No relevant answer	0.0%		0.0%		25.0%	1	0.0%		0.0%				0.0%		1.7%	1
Missing	29.4%	5	25.0%	2	0.0%		28.6%	2	0.0%		100%	1	70.6%	12	37.9%	22
	100.0%	17	100%	8	100%	4	100%	7	100%	4	100%	1	100%	17	100%	58
																n=6
																%
																16.7%
																0.0%
																66.7%
																4
																0.0%
																0.0%
																16.7%
																1
																6

Conclusions

Most respondents are of the opinion that nurses should collaborate in a team with the doctor, pharmacist and other healthcare professionals where pharmaceutical care is involved. It was also stated that nurses would probably be more familiar with the term 'medication management' than 'pharmaceutical care'. Nurses themselves seem to rate their role in pharmaceutical care as crucial to a considerably higher extend than the other stakeholders. The following roles of nurses were described:

- 'Managing role, follow instructions to help patient use their medicines right, responsible for distributing drugs to inpatients according to the instructions of physicians, monitoring patients, listening to patients and communicating needs/problems, appropriate information to the patient related to healthcare and medical education.'
- 'Nurses focus on the practical aspects of medicines use and pay less attention to the appropriateness.'
- 'In conjunction with the doctor and pharmacist, the nurse has an important role in contributing to, and delivering therapeutic care plans. If this is truly a team-approach in the interest of the patient, the nurse, as a care-giver should also be in a position to raise issues regarding the care plan with the doctor and pharmacist.'

Other healthcare professionals

	Sectors																	Total	Experts
	Pharm	n	PH	n	MD	n	Pat	n	Nurs	n	Others	n	Unknown	n			n=58	%	n=6
Key/important role	5.9%	1	12.5%	1	25.0%	1	0.0%		25.0%	1			0.0%				4	6.9%	0.0%
Cooperative, together in a team	23.5%	4	0.0%		25.0%	1	0.0%		0.0%				17.6%	3			8	13.8%	0.0%
Specific role: see conclusion	5.9%	1	0.0%		0.0%		28.6%	2	0.0%				0.0%				3	5.2%	16.7%
Passive/minimal	0.0%		0.0%		0.0%		0.0%		0.0%				0.0%					0.0%	
None	0.0%		0.0%		0.0%		0.0%		0.0%				5.9%	1			1	1.7%	0.0%
Missing	64.7%	11	87.5%	7	50.0%	2	71.4%	5	75.0%	3	100%	1	76.5%	13			42	72.4%	83.3%
	100.0%	17	100%	8	100%	4	100%	7	100%	4	100%	1	100%	17			58	100%	100%

Conclusions

Most respondents do not consider that other healthcare professionals play a crucial role in pharmaceutical care. However, other healthcare professionals such as physiotherapists and social workers may provide support as part of a pharmaceutical care team for patients with special needs. A relatively high number of respondents did not answer this question, suggesting that they felt the role of other healthcare professionals in pharmaceutical care less prominent.

Patients

[illegible]

Conclusions

Most stakeholders consider that the patient has an important role to play. The role of the informed, empowered patient was critical and very important, as patients 'have to make choices and ask questions'. Respondents reported the following regarding the patients' role:

- ‘Patients’ role is critical however the level of involvement may vary and will dependent on patients’ knowledge and education from healthcare providers.’
- ‘With the pharmaceutical care approach, the patient is considered as an active partner in care planning and as the ultimate decision maker. Pharmaceutical care creates a bond, therapeutic relationship, between the two parties: the patient and the pharmacist.’
- ‘The patient is central to any therapeutic care plan. They must feel empowered in their own care and treatment. The patient should always feel that they can ask for information, contribute their views and be consulted on the development, delivery and monitoring of their own therapeutic regimen and care plan.’
- ‘Inform doctor about all medicines taken, inclusive of OTCs, concordance; confer with the doctor or pharmacists, report adverse effects.’
- ‘Patients have a role in ensuring the safety and effectiveness of their medicines and require additional information and support to do this better.’

However, the patients' associations' replies pointed out that patients in reality may face a situation that is far from ideal:

- 'The patient's role is to passively take medicines according to the above actions'.
- 'Patients are not allowed to speak.'

Carers

	Sectors																	Total	Experts	
	Pharm	n	PH	n	MD	n	Pat	n	Nurs	n	Others	n	Unknown	n	%	%	%	n	n=58	%
Key/important role	23.5%	4	12.5%	1	25.0%	1	14.3%	1	50.0%	2			5.9%	1	17.2%		0.0%	1	10	0.0%
Cooperative, together in a team	17.6%	3	12.5%	1	25.0%	1	0.0%		0.0%				0.0%		8.6%		0.0%		5	0.0%
Specific role: see conclusion	0.0%		0.0%		25.0%	1	0.0%		0.0%				0.0%		1.7%		50.0%	1	1	3
Passive/minimal	0.0%		0.0%		0.0%		28.6%	2	25.0%	1			0.0%		5.2%		0.0%		3	
No relevant answer	5.9%	1	0.0%		0.0%		0.0%		0.0%				0.0%		1.7%		0.0%		1	
Missing	52.9%	9	75.0%	6	25.0%	1	57.1%	4	25.0%	1	100%	1	94.1%	16	65.5%		50.0%	38	38	50.0%
	100.0%	17	100%	8	100%	4	100%	7	100%	4	100%	1	100%	17	100%		100%	58	58	100%

Conclusions

Only few replies dealt with the role of carers which was in general considered as less important than that of the patient. However, some respondents do see an important role for carers:

- ‘Unfortunately often forgotten but very important for providing practical information about patient’s and carer’s particular circumstances and needs.’
- ‘Carers also have a very involved role in the care of the patient. They should also be given the opportunity to raise issues regarding the care plan with the doctor and pharmacist.’
- ‘Correct administration of medicines, monitoring adverse effects, confer with the doctor.’

Q.8 In your view, which outcome has the application of the concept of pharmaceutical care, statement 1, on the patients' medicine therapy?*

[illegible]

Conclusions

The majority of respondents, independent of the sector they represent, are of the opinion that the concept of pharmaceutical care has a good outcome on the patient's medicine therapy. Doctors and patients' associations seem to be less sure about the outcomes of pharmaceutical care.

Some respondents think that there is a probability that outcomes will be good. Very few point to the lack of scientific evidence and state that it is not yet possible to define the outcome.

* See Appendix, survey questionnaire

Q.9 Please describe the barriers for implementing in practice the concept of pharmaceutical care, statement 1*, in your country.

	Sectors	n	PH	n	MD	n	Pat	n	Nurs	n	Others	n	Unknown	n	Total	%	n=58	Experts	%	n=6
	Pharm																			
Lack of cooperation/communication with doctors	11.8%	2	12.5%	1	50.0%	2	14.3%	1	50.0%	2			17.6%	3	19.0%		11	0.0%		
Lack of finance/time	5.9%	1	12.5%	1	25.0%	1	0.0%		0.0%				0.0%		5.2%		3	16.7%	1	1
Lack of awareness/national plan/legal basis	5.9%	1	25.0%	2	0.0%		0.0%		0.0%				5.9%	1	6.9%		4	33.3%	2	2
Lack of training/skills/knowledge	11.8%	2	12.5%	1	0.0%		0.0%		25.0%	1			0.0%		6.9%		4	0.0%		
Combination of 2 or more items from above	29.4%	5	0.0%		0.0%		28.6%	2	25.0%	1			17.6%	3	19.0%		11	0.0%		
Other barriers: see conclusion*	11.8%	2	25.0%	2	0.0%		0.0%		0.0%				5.9%	1	8.6%		5	33.3%	2	2
No barriers	0.0%		0.0%		25.0%	1	0.0%		0.0%				0.0%		1.7%		1	0.0%		
No relevant answer	0.0%		0.0%		0.0%		42.9%	3	0.0%				0.0%		5.2%		3	0.0%		
Missing	23.5%	4	12.5%	1	0.0%		14.3%	1	0.0%		100%	1	52.9%	9	27.6%		16	16.7%	1	1
	100.0%	17	100%	8	100%	4	100%	7	100%	4	100%	1	100%	17	100%		58	100%		6

Conclusions

The respondents describe a large number of barriers to the implementation of pharmaceutical care in practice. One of the respondents summed up the situation in many countries: 'The expectations of the healthcare system, other healthcare professionals and pharmacists themselves is that their only role is to dispense prescriptions accurately.' To change the attitudes towards the implementation of pharmaceutical care amongst the healthcare professionals and the patients and their carers, a paradigm shift is needed, as described by another respondent: 'There is a need of a formal structure for the development, delivery and monitoring of "pharmaceutical" and therapeutic care plans for patients, where the roles, responsibilities and relationship between all the stakeholders, i.e. prescribers, pharmacists, nurses, other health professionals, patients and their carers.' And 'we need agreement amongst all stakeholders as to how "pharmaceutical care" is understood and interpreted, how it is delivered and monitored, what approach is taken, how accountability is apportioned, what communication mechanisms are put in place and how the quality of care provided is measured and assured.'

Other more specific barriers mentioned are: lack of

- technological support (pharmacy software)
- time investment by pharmacists
- knowledge and adequate training (e.g. concerning communication)
- responsibility for pharmacotherapy taken by pharmacists
- collaboration between pharmacist, prescribers and other healthcare professionals
- sharing of data between healthcare providers.
- reimbursement of pharmaceutical care services
- patient education
- pharmacies
- quality indicators
- health literacy among the public.

* See Appendix, survey questionnaire

Q.10 Do healthcare professionals share patients' records in order to promote the use of the concept?

	Sectors	n	PH	n	MD	n	Pat	n	Nurs	n	Others	n	Unknown	n	Total	n=58
Yes	Pharm	5	12.5%	1	25.0%	1	14.3%	1	0.0%				5.9%	1	15.5%	9
No		7	37.5%	3	75.0%	3	57.1%	4	50.0%	2			35.3%	6	43.1%	25
Do not know		2	37.5%	3	0.0%		28.6%	2	50.0%	2			17.6%	3	20.7%	12
Missing		3	12.5%	1	0.0%		0.0%		0.0%		100%	1	41.2%	7	20.7%	12
		17	100%	8	100%	4	100%	7	100%	4	100%	1	100%	17	100%	58

Conclusions

Respondents from only 2 countries replied that patients' records are shared. In 7 countries records are not shared. In 6 countries the answers of the different respondents varied. Surprisingly, pharmacists more often stated that records are shared; this might apply more particularly to the hospital situation. The answers suggest that the sharing of patients' records is not common practice throughout Europe; this is confirmed by the experts.

- 'Physicians and pharmacists will start using the 'electronic medication record'.'
- 'Healthcare professionals share patient's records at the occasion of transfer or finding of drug-related problems.'
- 'Patients' records are shared in the hospital setting but not in the community setting.'
- 'Little if any sharing occurs currently. An electronic e-prescription and patient record system would facilitate sharing. Telephones, e-mails and letters are used to share information currently. Some transfer of test results via information technology is possible.'
- 'Sharing of patients' records occurs in an unstructured and reactive way where one professional seeks information from another in order to clarify a matter in relation to the patient's care. The sharing of a patient record does not occur in a proactive way amongst healthcare professionals in order to develop a structured approach to the patient's therapeutic care plan.'

32

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100

Q.12 What is the priority of the concept of pharmaceutical care, see statement 1*, in your government policy on medication therapy? Could you give reasons for the give priority of the concept of pharmaceutical care?

	Sectors	PH	n	PH	n	MD	n	Pat	n	Nurs	n	Others	n	Unknown	n	Total		Experts
	Pharm	n		n		n		n		n		n		n		%	n=58	%
High priority	5.9%	1	37.5%	3	25.0%	1	0.0%	0.0%	0.0%	0.0%	0.0%		1	5.9%	1	10.3%	6	16.7%
Middle priority	35.3%	6	50.0%	4	25.0%	1	28.6%	2	50.0%	2	0.0%		3	17.6%	3	31.0%	18	0.0%
Low priority	0.0%		0.0%		0.0%		0.0%		0.0%				2	11.8%	2	3.4%	2	0.0%
Under consideration for setting future priorities	29.4%	5	0.0%		50.0%	2	42.9%	3	25.0%	1			2	11.8%	2	22.4%	13	33.3%
No priority	5.9%	1	0.0%		0.0%		0.0%		0.0%				2	11.8%	2	5.2%	3	33.3%
Do not know	5.9%	1	0.0%		0.0%		28.6%	2	25.0%	1				0.0%		6.9%	4	16.7%
Missing	17.6%	3	12.5%	1	0.0%		0.0%		0.0%			100%	1	41.2%	7	20.7%	12	0.0%
	100.0%	17	100%	8	100%	4	100%	7	100%	4	100%	1	17	100%	58	100%	6	100%

Conclusions

Most stakeholders report that pharmaceutical care in medication therapy has middle priority or is taken into consideration in setting future priorities. Assigning pharmaceutical care a less than high-priority status is also mentioned as one of the barriers to the implementation of pharmaceutical care.

Some respondents explained their rating of priority:

For a 'high priority' reply:

- 'A guide for good pharmacy practices will be published shortly. The possible impact on public health and government budget has urged policy makers to endorse the concept of pharmaceutical care.'
- 'It is a high priority item but slow to develop in practice. Organisational, technical, legal, industrial relation barriers can slow progress. However it is necessary to work with what you have in the interim.'
- 'In our country are ongoing reforms in the health sector, in order to improve health protection there are undertaken a lot of activities including pharmaceutical care.'

For a 'middle priority' reply:

- 'Poor resources.'
- 'Community pharmacists are paid additional money to undertake medication use review for selected patients - which is a start.'
- 'In the ongoing discussions on the reform of healthcare pharmaceutical care is seen as a suitable possibility to reduce costs.'
- 'Recently, in new legislation, pharmacists are now legally required to carry out a review of medicine therapy and counsel and advise the patient appropriately on their therapeutic care regimen. It is expected that this new legal requirement will lead to a more structured approach to patient care and it is hoped that this will also lead to a formal structured implementation of a "pharmaceutical care" approach as outlined in statement 1.'

For an 'under consideration for setting future priorities':

- 'This issue is not treated at government level, but through negotiations between health professional organisations and social insurance.'
- 'The concept is in our legislation but not practised.'
- 'Recent patient safety report with section on medication safety report recommendations not implemented as yet.'

* See Appendix, survey questionnaire

Q.13 Is there a legal basis, health strategy or policy document for the implementation of the concept of pharmaceutical care, see statement 1*, in your country?

	Sectors														Total	Experts	
	Pharm	n	PH	n	MD	n	Pat	n	Nurs	n	Others	n	Unknown	n	%	%	n=58
Yes	35.3%	6	50.0%	4	25.0%	1	14.3%	1	25.0%	1			29.4%	5	31.0%	33.3%	2
No	17.6%	3	37.5%	3	50.0%	2	42.9%	3	25.0%	3			17.6%	3	25.9%	50.0%	3
Do not know	29.4%	5	0.0%		25.0%	1	42.9%	3	50.0%	3			11.8%	2	22.4%	16.7%	1
Missing	17.6%	3	12.5%	1	0.0%		0.0%		0.0%		100%	1	41.2%	7	20.7%	0.0%	
	100.0%	17	100%	8	100%	4	100%	7	100%	4	100%	1	100%	17	100%	100%	6

Conclusions

Public health authorities and pharmacists more often than other stakeholders are of the opinion that there is a legal basis; health strategy or policy document for the implementation of pharmaceutical care. A respondent informed that the term 'pharmaceutical care' is defined in Art. 4 § 2bis of RD 78 of 10/11/1967 and that a guide to "Good Pharmacy Practices" is to be published shortly.

Web links indicated by certain respondents:

- www.moh-HSMP.gov.mk; Health strategy 2020
- http://www.dohc.ie/legislation/statutory_instruments/pdf/si20080488.pdf?direct=1. In addition, the report published in April 2008 entitled 'Advancing Clinical Pharmacy Practice to Delivery Better Patient Care and Added Value Services' sets out new policy and is intended to chart a course for the future the development of practice and the provision of pharmaceutical care in the interests of patients.
- A white paper regarding correct drug use: www.regjeringen.no/nb/dep/hod/dok/regpubl/stmeld/20042005/Stmeld-nr-18-2004-2005-.html? and also regarding the role of health personal: <http://www.lovdato.no/all/nl-19990702-064.html>
- www.gyogygond.hu
- [www.apotheker.or.at->Themenbereiche->Information der Rechtsabteilung->Apothekenrelevante Gesetze und Verordnungen](http://www.apotheker.or.at->Themenbereiche->Information%20der%20Rechtsabteilung->Apothekenrelevante%20Gesetze%20und%20Verordnungen). http://www.bmgfj.gv.at/cms/site/presse_detail.html?channel=CH0616&doc=CMS1212586753323.

* See Appendix, survey questionnaire

Q.14 In your country, are there contracts between health professionals and employers, insurers and health service providers to implement the concept of pharmaceutical care, see statement 1? (If you have replied 'yes' to one or both of the above questions (Q13, 14), please send to the Secretariat the web link of the legal basis, health strategy or policy document.)

[illegible]

Conclusions

Although public health authorities report the existence of contracts between health professionals and other stakeholders to implement the concept of pharmaceutical care, the majority of professional stakeholders and patients are not familiar with contracts. The contracts that are mentioned do not refer to the whole field of pharmaceutical care, but merely to some specific activities:

- ‘Some hospital and community pharmacists also operate (in) clinics overseeing the use of medicines and some have independent and supplementary prescribing rights.’
- ‘In a few cases, and in relation to co-payment matters, agreements exist between pharmacist and sick funds (e.g. oxygen therapy at home, methadone substitution therapy). www.faggg.be and www.inami.fgov.be’

* See Appendix, survey questionnaire

III. Management of medicine therapy and use of medication as a well-controlled chain of subsequent and parallel activities and processes

Q.15 How do you rank the importance of the medicine therapy and medication chain?

	Sectors															Total	Experts	
	Pharm	n	PH	n	MD	n	Pat	n	Nurs	n	Others	n	Unknown	n	%	n=58	%	n=6
High	64.7%	11	75.0%	6	75.0%	3	71.4%	5	100.0%	4			41.2%	7	62.1%	36	100.0%	6
Medium	5.9%	1	0.0%		25.0%	1	14.3%	1	0.0%				0.0%		5.2%	3	0.0%	
Low	5.9%	1	12.5%	1	0.0%		0.0%		0.0%				5.9%	1	3.4%	2	0.0%	
None	0.0%		0.0%		0.0%		0.0%		0.0%				5.9%	1	1.7%	1	0.0%	
Do not know	0.0%		0.0%		0.0%		14.3%	1	0.0%				0.0%		1.7%	1	0.0%	
Missing	23.5%	4	12.5%	1	0.0%		0.0%		0.0%		100%	1	47.1%	8	25.9%	15	0.0%	
	100.0%	17	100%	8	100%	4	100%	7	100%	4	100%	1	100%	17	100%	58	100%	6

Conclusions

A majority of respondents considered the management of the medicine therapy and medication as being highly important. It is interesting that this does not automatically lead to the implementation of pharmaceutical care. However, this general feeling of importance is an important pre-requisite for putting the implementation of pharmaceutical care high on the agenda.

Q. 16 Is the quality of the management of medicine therapy and medication chain in your country being measured on a routine basis?
(see Q. 17)

Q. 17 If the answer to Q 16 was 'yes', how is the quality of the management of the medicine therapy and medication chain measured?

	Sectors	n	PH	n	MD	n	Pat	n	Nurs	n	Others	n	Unknown	n	Total	n=58	Experts	n=6
Yes	Pharm	1	37.5%	3	0.0%		14.3%	1	0.0%				5.9%	1	10.3%	6	16.7%	1
No		11	50.0%	4	100.0%	4	57.1%	4	75.0%	3			35.3%	6	55.2%	32	66.7%	4
Do not know		1	0.0%		0.0%		28.6%	2	25.0%	1			11.8%	2	10.3%	6	16.7%	1
Missing		4	12.5%	1	0.0%		0.0%		0.0%		100%	1	47.1%	8	24.1%	14	0.0%	
		17	100%	8	100%	4	100%	7	100%	4	100%	1	100%	17	100%	58	100%	6

Conclusions

The majority of the respondents stated either that the quality of the management of medicine therapy and medication chain is not measured on a routine basis, or that they do not know if it was measured. The answers from respondents who reported that the quality of management of the medicine therapy and medication chain is measured mentioned the following ways in which this is done in their country:

- the use of therapeutic guidelines, protocols and standards by the different professions
- drug utilisation statistics
- mortality, morbidity rates and (re-)hospitalisation rates
- adverse drug reporting systems
- total number of days of sick leave

The answers indicate that the quality of the management of the medicine therapy and medication chain is not routinely measured. Although the availability of guidelines, protocols and standards and statistics are important pre-requisites for measuring quality, this does not mean that the quality is actually measured.

Q.18 How do you rank the importance of the safe use of medicines for overall patient safety?

	Sectors	n	PH	n	MD	n	Pat	n	Nurs	n	Others	n	Unknown	n	Total	n=58	Experts	n=6
	Pharm	13	87.5%	7	100.0%	4	100.0%	7	100.0%	4			41.2%	7	72.4%	42	100.0%	6
High																		
Medium	76.5%		0.0%		0.0%		0.0%		0.0%				5.9%	1	1.7%	1	0.0%	
Low	0.0%		0.0%		0.0%		0.0%		0.0%				0.0%		0.0%		0.0%	
None	0.0%		0.0%		0.0%		0.0%		0.0%				0.0%		0.0%		0.0%	
Do not know	0.0%		0.0%		0.0%		0.0%		0.0%				0.0%		0.0%		0.0%	
Missing	23.5%	4	12.5%	1	0.0%		0.0%		0.0%		100%	1	52.9%	9	25.9%	15	0.0%	
	100.0%	17	100%	8	100%	4	100%	7	100%	4	100%	1	100%	17	100%	58	100%	6

Conclusions

The majority of the respondents considered the safe use of medicines for overall patient safety to be highly important. Like the high ranking of the importance of the management of the medicine therapy and the medication chain, this does not automatically lead to the implementation of pharmaceutical care. However, this general feeling of importance is an important pre-requisite for putting the implementation of pharmaceutical care high on the agenda.

Q. 19 In which areas of the national healthcare system is patient safety measured in your country?

The replies on this questionnaire not tabulated as the respondents seem to interpret this question in different ways. Some respondents refer to areas of healthcare such as primary healthcare or hospitals, while others refer to authorities in charge of overseeing pharmacovigilance such as the Ministry of Health. At the international level, it is known that many countries in Europe participate in adverse drug reaction reporting systems such as that operated by the WHO [45]. However, it is also known that the rate of under-reporting is high [46]. Spontaneous adverse drug reporting is therefore not a systematic method of measuring patient safety. One respondent made an interesting remark: 'Although drug safety issues associated with the intrinsic adverse events connected with medicines are measured and recorded formally, there is no formal system for monitoring medication errors or the safety and quality of patient counselling regarding the safe use of medicines or the safety and quality of patient care associated with medicines.'

Q. 20 Is the safety of medication use measured in your country?
(see Q. 21)

Q. 21 If the answer to Q. 20 was 'yes', how is the safety of medication use measured?

	Sectors	n	PH	n	MD	n	Pat	n	Nurs	n	Others	n	Unknown	n	Total	n=58	Experts	n=6
Yes	Pharm	5	50.0%	4	75.0%	3	28.6%	2	50.0%	2		1	5.9%	1	29.3%	17	33.3%	2
No		4	25.0%	2	25.0%	1	28.6%	2	25.0%	1		6	35.3%		27.6%	16	66.7%	4
Do not know		4	12.5%	1	0.0%		42.9%	3	25.0%	1		1	5.9%		17.2%	10	0.0%	
Missing		4	12.5%	1	0.0%		0.0%		0.0%		100%	9	52.9%	1	25.9%	15	0.0%	
		17	100%	8	100%	4	100%	7	100%	4	100%	17	100%	1	100%	58	100%	6

Conclusions

The majority of respondents answered that the safety of medication use is not measured in their country or they did not know if it was measured. The respondents who said that safety is measured refer to the availability of national drug reporting systems and to the presence of national or local prescribing guidelines to improve safe prescribing. (see also conclusions of Q. 19).

Q.22 How could the measurement of patient safety be improved?

	Sectors															Total*		Experts	
	Pharm	n	PH	n	MD	n	Pat	n	Nurs	n	Others	n	Unknown	n	%	n=58	%	n=6	
Implementation of pharmaceutical care	29.4%	5	25.0%	2	0.0%		0.0%		0.0%				11.8%	2	15.5%	9	16.7%	1	
Improve electronic data management and sharing	11.8%	2	37.5%	3	50.0%	2	42.9%	3	50.0%	2			0.0%		20.7%	12	16.7%	1	
Improving safety culture	5.9%	1	12.5%	1	25.0%	1	0.0%		0.0%				0.0%		5.2%	3	16.7%	1	
Development of quality indicators	11.8%	2	12.5%	1	0.0%		14.3%	1	50.0%	2			0.0%		10.3%	6	16.7%	1	
Pharmacovigilance reporting	23.5%	4	12.5%	1	25.0%	1	0.0%		0.0%				0.0%		10.3%	6	16.7%	1	
Education	0.0%		0.0%		0.0%		0.0%		0.0%				0.0%		0.0%		16.7%	1	
Do not know	0.0%		0.0%		0.0%		28.6%	2	0.0%				11.8%	2	6.9%	4	0.0%		
No relevant answer	0.0%		0.0%		0.0%		0.0%		0.0%				11.8%	2	3.4%	2	0.0%		
Missing	23.5%	4	25.0%	2	0.0%		14.3%	1	0.0%		100%	1	64.7%	11	32.8%	19	0.0%		
		18		10		4		7		4				17		61		6	

Conclusions

In particular respondents from pharmacists' associations are of the opinion that implementing the concept of pharmaceutical care could improve patient safety. Some respondents suggest that the first step in their country is still to make patient safety one of the priorities in the national healthcare system. But even in countries where patient safety is high on the political agenda 'it remains necessary to promote a culture of medication safety reporting in all areas of the health sector e.g. manufacturers, pharmacies, hospitals, medical practitioners, other health professionals, social services, regulatory authorities'. Respondents from public health and patients' associations considered the improvement of electronic data management and sharing of data as important. The majority of the issues reported were mentioned earlier as being essential for the development of pharmaceutical care.

* Some respondents gave replies belonging to more than one category.

V. Patient participation in achieving good medication outcomes

Q.23 How do you rank the importance of joint decision-making between the patient and the healthcare professional for good outcomes? Please indicate areas for improvement, if applicable:

	Sectors	n	PH	n	MD	n	Pat	n	Nurs	n	Others	n	Unknown	n	Total	n=58	Experts	n=6
	Pharm	10	75.0%	6	100.0%	4	85.7%	6	100.0%	4			23.5%	4	58.6%	34	83.3%	5
High																		
Medium	58.8%	2	12.5%	1	0.0%		14.3%	1	0.0%				5.9%	1	8.6%	5	16.7%	1
Low	11.8%	1	0.0%		0.0%		0.0%		0.0%				5.9%	1	3.4%	2	0.0%	
None	5.9%																	
	0.0%		0.0%		0.0%		0.0%		0.0%				0.0%		0.0%		0.0%	
Do not know	0.0%		0.0%		0.0%		0.0%		0.0%				0.0%		0.0%		0.0%	
Missing	23.5%	4	12.5%	1	0.0%		0.0%		0.0%		100%	1	64.7%	11	29.3%	17	0.0%	
	100.0%	17	100%	8	100%	4	100%	7	100%	4	100%	1	100%	17	100%	58	100%	6

Conclusions

Most of the respondents considered the importance of joint decision-making between the patient and the healthcare professional for good outcomes as being high. Areas of improvement reported by certain respondents are:

- Improve communication skills like motivational interviewing healthcare professionals
- Improvement of quality of information to patients
- More involvement of the patient in the decision-making process
- More time for healthcare professionals to spend with patients
- More privacy

These conclusions are also raised in scientific literature.

Specific suggestions

- The development of protocols (e.g. within disease management programmes) and documentation of how many patients are treated according to these protocols
- The measurement of patient outcomes, including compliance with therapy and patient/carer satisfaction
- Patient interviews or questionnaires after consultations

[illegible]

Q.26 How do you assess the impact of joint decision-making?

	Sectors	n	PH	n	MD	n	Pat	n	Nurs	n	Others	n	Unknown	n	Total	n=58	Experts	%	n=6
Not possible/available	Pharm	23.5%	4	50.0%	4	0.0%	28.6%	2	0.0%				5.9%	1	19.0%	11	16.7%	16.7%	1
Specific examples: see below		11.8%	2	12.5%	1	25.0%	42.9%	3	25.0%	1			5.9%	1	15.5%	9	33.3%	33.3%	2
No relevant answer		29.4%	5	25.0%	2	75.0%	0.0%	3	50.0%	2			5.9%	1	22.4%	13	0.0%	0.0%	
Do not know		11.8%	2	0.0%		0.0%	14.3%	1	25.0%	1			5.9%	1	8.6%	5	50.0%	50.0%	3
Missing		23.5%	4	12.5%	1	0.0%	14.3%	1	0.0%		100%	1	76.5%	13	34.5%	20	0.0%	0.0%	
		100.0%	17	100%	8	100%	100%	4	100%	4	100%	1	100%	17	100%	58	100%	100%	6

Conclusions

Most respondents are not able to give specific examples of how the impact of joint decision-making may be assessed. The examples that are given suggest this is not done on a routine basis but as part of research (e.g. by using questionnaires and patient interviews following consultations).

Respondents seem to interpret this question as meaning 'Why is joint decision-making important?', since they give the following answers:

- 'Improves patient concordance and motivates patient to take self-responsibility'
- 'Beneficial for patients protection and therapy outcomes'
- 'Less drug sold, better policies, more satisfied customer with the health system'
- 'It makes visible the patient's role in the improvement of her/his health'

Q. 27 If the impact of joint decision making in medication therapy was not assessed in your country, how would you propose measuring it?

	Sectors	n	PH	n	MD	n	Pat	n	Nurs	n	Others	n	Unknown	n	Total	%	Experts	%	n=58	n=6
Not possible/available	Pharm	2	25.0%	2	25.0%	1	0.0%		0.0%				0.0%		8.6%	5	16.7%	1		
	Specific suggestions see below	7	50.0%	4	25.0%	1	14.3%	3	75.0%				5.9%	1	29.3%	17	33.3%	2		
No relevant answer		2	12.5%	1	50.0%	2	28.6%	1	25.0%				0.0%		13.8%	8	0.0%			
Do not know		2	0.0%		0.0%	3	42.9%		0.0%				11.8%	2	12.1%	7	33.3%	2		
Missing		4	12.5%	1	0.0%		14.3%	1	0.0%		100%	1	82.4%	14	36.2%	21	16.7%	1		
		17	100%	8	100%	4	100%	7	100%	4	100%	1	100%	17	100%	58	100%	6		

Conclusions

Respondents' suggestions essentially pointed in the same directions as for the previous questions 24, 25 and 26: 'It is difficult to assess the impact of joint decision-making in a meaningful way.'

The same comments as in question 25 are made:

- The development of protocols (e.g. within disease management programmes) and documentation of how many patients are treated according to these protocols
- The measurement of patient outcomes, including compliance with therapy and patient/carer satisfaction
- Patient interviews or questionnaires after consultations

One respondent also suggested collecting national statistics on health literacy and patient involvement in health services.

VI. Ensuring the best possible outcome

Q. 28 *What systems do you have in place to ensure good, clinically effective outcomes of medication at population level?*

	Sectors	n	PH	n	MD	n	Pat	n	Nurs	n	Others	n	Unknown	n	Total*	n=58	Experts
	Pharm	4	25.0%	2	50.0%	2	14.3%	1	0.0%				0.0%		15.5%	9	n=6
Not possible/available	23.5%																3
Do not know	0.0%		0.0%		0.0%		14.3%	1	50.0%	2			5.9%	1	6.9%	4	33.3%
No relevant answer	5.9%	1	12.5%	1	25.0%	1	14.3%	1	0.0%				0.0%		6.9%	4	0.0%
Specific systems: see conclusions	47.1%	8	50.0%	4	25.0%	1	42.9%	3	25.0%	1			5.9%	1	31.0%	18	16.7%
Do not understand the question	0.0%		0.0%		0.0%		0.0%		25.0%	1			0.0%		1.7%	1	0.0%
Missing	23.5%	4	12.5%	1	0.0%		14.3%	1	0.0%		100%	1	88.2%	15	37.9%	22	0.0%
	100.0%	17	100%	8	100%	4	100%	7	100%	4	100%	1	100%	17	100%	58	100%
																	6

Conclusions

A third of the respondents is aware of the availability of systems ensuring good, clinically effective medication outcomes at the population level. One respondent summed up the answers given by many respondents: 'The systems in place at present are primarily input rather than outcome focussed. Regulatory systems exist to set standards for the quality of medicines, the standard of services, facilities, equipment, the education and training of health professionals. The systems set standards for the quality of medication therapy but do not really measure the clinical outcomes. National statistics are maintained regarding population health however, these are not specifically correlated to inputs.'

Examples of the type of systems that are included in this answer are:

- Market authorisation of medicines by authorities
- Specially structured reimbursement system
- Adverse drug reaction reporting
- Promotion of evidence-based medicine and pharmacy service
- Use of protocols for care in certain cases; prescribing policies/ protocols, e.g. for methadone substitution
- (Continuous) training for professionals

* Some respondents gave replies belonging to more than one category.

Q.29 Please describe 3 priorities for your country in the fields of safety and quality of pharmaceutical care.
(See Q. 30.)

Q.30 Please describe how the above 3 priorities in the fields of safety and quality of pharmaceutical care are measured and assessed.

	Sectors															Total		Experts
	Pharm	n	PH	n	MD	n	Pat	n	Nurs	n	Others	n	Unknown	n	%	n=58	%	n=6
Electronic patient data and sharing between professionals	17.6%	3	62.5%	5	50.0%	2	28.6%	2	50.0%	2		2	5.9%	1	25.9%	15	16.7%	1
Better education of healthcare professionals	17.6%	3	25.0%	2	25.0%	1	0.0%		25.0%			1	5.9%	1	13.8%	8	0.0%	
More patient centred approach	5.9%	1	50.0%	4	0.0%		0.0%		75.0%			3	11.8%	2	17.2%	10	16.7%	1
Improved drug regulation and policy	17.6%	3	12.5%	1	50.0%	2	28.6%	2	50.0%	2		2	0.0%		17.2%	10	0.0%	
More attention to pharmacovigilance and patient safety	11.8%	2	25.0%	2	25.0%	1	0.0%		0.0%			2	11.8%	2	12.1%	7	33.3%	2
Implementing (specific) pharmaceutical care services	47.1%	8	12.5%	1	75.0%	3	14.3%	1	25.0%	1		1	0.0%		24.1%	14	0.0%	
Financial and other resources	11.8%	2	12.5%	1	25.0%	1	42.9%	3	25.0%	1		2	11.8%	2	17.2%	10	0.0%	
Miscellaneous	0.0%		0.0%		25.0%	1	42.9%	3	50.0%	2			0.0%		10.3%	6	0.0%	
Not measured	23.5%	4	25.0%	2	0.0%		14.3%	1	25.0%	1		1	0.0%		13.8%	8	33.3%	2
Do not know	0.0%		0.0%		0.0%		14.3%	1	25.0%	1		1	0.0%		3.4%	2	33.3%	2
Missing	23.5%	4	25.0%	2	0.0%		14.3%	1	0.0%	1	100%	1	88.2%	15	39.7%	23	16.7%	1
		30		20		11		14		14	100%	1		23		113		9

Conclusions

Questions 29 and 30 will be discussed together as the answers are related. Although pharmacists particularly but also other stakeholders do give priority to the implementation of (specific) pharmaceutical care, the majority of respondents refer to more basic priorities that could be interpreted as a pre-requisite for the development of pharmaceutical care, such as:

- The availability of complete medication records and the use of these records for medication surveillance and the sharing of information among healthcare providers
- Better training for healthcare providers, particularly in the field of evidence-based medicine
- A more patient-centred approach in healthcare
- Improving the regulatory system of pharmaceuticals and pharmacy services
- More attention to medication safety and the implementation of pharmacovigilance
- More financial resources and sufficient staff to meet healthcare needs

The replies are listed in detail in Table 5.

Table 5

	Priority (Q. 29)	How priority is measured and assessed (Q. 30)
PH	Availability of complete “pharmaceutical records” of the patient, with all dispensed health products documented	Completeness of local patient’s records
PH	To use this data to manage Drug-Related Problems	Better management of drug-related problems (number detected, improved compliance, etc)
MED	Implementation of electronic medication dossier all over the country	
NOT KNOWN	Intake patients dossier and link doctor and pharmacist computer networks	Build concerted database for each patient
PHARM	Medical safety; checking for possible interactions of medicines, including ‘over the counter medicines’ (OTCs), to be performed by the doctor at the time of prescribing and not after the event	System that produces automatic warnings in cases of severe medicine interactions
PHARM	Drug interactions	
PAT	Accept the e-card (electronic health management)	No paper – all is directed and shown in safety computers
MED	Implementation of Electronic Prescription appliance for prescribers	
PHARM	Using good software with drug-drug and drug-food interactions	Prevention of drug-related problems, safety medication – individual project – faculties, insurance companies
PH	Establishment of an effective e-health strategy to facilitate management and monitoring of the health system	An ongoing strategy is being developed in conjunction with EC e- health strategy.
PH	To share (a part of) this data among pharmacists and other healthcare professionals	Number of shared records – completeness of shared records – evaluation of good use
PHARM	Sharing patient’s record or medication in pharmacy	Avoidance of drug-related problems before false medication, preventing of medication errors – hospital pharmacists, clinical pharmacists
NURS	Co-operation between health professionals	Meetings
NURS	Recognition of inter-professional collaboration for pharmaceutical care	Existence of inter-professional initiatives and participation at national and local levels
PH	More posts for clinical pharmacists in hospitals	Not in place hence not measured and assessed
PAT	Your standard doctor has to know all your steps	My doctor should know if I take Viagra or if my orthopaedic specialist doctor prescribes cortisone, although I self-administer insulin
PHARM	Continuous training for healthcare professionals	Education – knowledge evaluation tests (not always)
UNKNOWN	Better-concerted training for doctors and pharmacists	Various aspects of training
MED	Change the training model for pharmacists	
PHARM	Continuous training programme for pharmacists and doctors	Assessing mistakes
NURS	Patient and HCP education for responsibilities	Survey among healthcare professionals and patients
PH	Provision of good-quality information to healthcare professionals and patients	Aim to try to comply with the recommendations of the pharmaceutical forum on this issue
PH	Feedback to doctors on rational drug use	

	Priority (Q. 29)	How priority is measured and assessed (Q. 30)
PH	More evidence-based use of medicines	Audits of practice National Institute for Health and Clinical Excellence (NICE) guidelines
NURS	Law	Outcomes
PHARM	Installing functional agency for medicinal products at country level	
PAT	Quality of drugs	Assessment of agency effectiveness
PHARM	Medicine quality control	Results of inspection checks
NURS	Avoid counterfeit medicine	
PH	More efficient treatment	Using health outcome data
PAT	High drug safety	Complete disclosure of revenues and expenditure of pharmaceutical industry a lot can be guessed from it
MED	Avoid false or quasi-true information via lay media	Establish guidelines for health professionals when speaking or writing in lay media
PHARM	Licensing of all healthcare establishments, including all pharmacies	All pharmacies will be licensed from 1 Jan. 2009, which will mean that matters relating to the pharmacy premises and services can be inspected, monitored and assessed.
MED	Strict controls in order to prevent the dispensing of prescription-only medicines without valid prescription	Strict controls by the competent authorities
UNKNOWN	Safety-belt programme tried out in one province	
PH	It is vital to gather and give feedback on information about near misses and other incidents via the Clinical Incident Reporting System and review risk management policies, standards and protocols continually as necessary. It is also important to be able to identify professional misconduct and to provide the powers to take appropriate action.	Finding effective ways to measure the safety and quality of pharmaceutical care is more or less difficult depending on where you are starting out from. Contracts and codes of practice are a starting point for both self-assessment and external assessment. Establishment of a good e-health strategy allows capture of information and potential for analysis if designed with this in mind and with an underlying strategy to improve the quality and safety of pharmaceutical care.
MED	Critical incidents	Building partnerships to stop the global cancer epidemics (PACT)
PHARM	Report all the medication errors to a central database	Medication errors (by patient, doctor and pharmacist)
PH	Reduced numbers of preventable adverse drug events	Patient safety incident report and adverse drug reaction reporting
PHARM	System of medication safety reporting and monitoring	Establishment of a system of reporting and monitoring medication safety issues and incidents
UNKNOWN	Improving system of pharmacovigilance	Build concerted data base for each drug- side effects and interactions
NURS	Provide clear and understandable information to clients	
UNKNOWN	Patients' benefit	Cure or at least slow the disease process
PHARM	Orientation of patient	Questionnaire
NURS	The collaboration of patients	Research
PHARM	Information for the patients	Patients become more compliant; not afraid to ask the pharmacist or other HC professional about their disease or medication.
PHARM	Patient/pharmacist relationship	
UNKNOWN	Patients' benefit	Statistical figures not available
PHARM	Patients' quality of life	None

	Priority (Q. 29)	How priority is measured and assessed (Q. 30)
PH	Higher compliance	Using patient questionnaires
NURS	Information to patients during joint decision-making	
NURS	Amending existing regulations for the rights of other Health Professionals (e.g. Nurses) to pharmaceutical care as you describe it	
PHARM	Introducing the concept	
PHARM	Implement national plan for pharmaceutical care	Negative clinical outcomes
PHARM	Implementation	
MED	Create consensus among stakeholders on what constitutes good pharmaceutical care	Facilitate inter-professional meetings
PHARM	Monitoring pharmaceutical care implementation in the community pharmacies (we don't have it yet in hospitals!!!)	The patients can get good services in all community pharmacies.
PHARM	Assessing the quality of pharmaceutical care delivery	Inspection of all pharmacies as mandated under new legislation will be required to assess compliance with regulatory requirements relating to the obligation on the pharmacist to provide appropriate advice on the supply of medicines and quality of pharmaceutical care delivered.
PH	Establishment of an effective overall medicines management policy starting in hospitals, nursing home and other institutions	Guidelines are being produced regarding care homes for the elderly and other institutions with medicine management sections and with quality criteria and standards to be complied with as appropriate.
PHARM	Diabetic services	
PAT	Asthma services	
MED	Repeat prescribing	Building partnerships to stop the global cancer epidemics (PACT)
PHARM	Medicine (use) review	
PHARM	Systems that provide medication reviews in ambulatory care	
MED	Protocols for responsibilities and safety for off-label prescriptions	
UNKNOWN	More long-term investment (less substitution)	
NURS	Human resources to implement good practice	Staffing ratios, skill mix
PAT	More people at work (less absence)	
PAT	Adequate number of therapists	
PAT	Economically independent doctors and pharmacists	
PH	Lower expenditures	With expenditures' data
PHARM	Financial stability of healthcare system	Availability of financial means
MED	Cost transparency	
PHARM	Pharmacoeconomy	None
UNKNOWN	Orientation of economy	International Organisation for Harmonisation (ISO)
PHARM	Orientation of staff	Accreditation
NURS	Prevent others to have "easy access" to medications and pharmaceutical care	
NURS	Alternative therapy	

	Priority (Q. 29)	How priority is measured and assessed (Q. 30)
PAT	Originals (Note: upon request for clarification the respondent indicated that branded medicines were meant)	
MED	Gather good data on the topic	Create a simple data recording tool
PAT	Pharmacists should take responsibility for recommending medicines to patients when self-medication is being applied	Legal definition of the pharmacist's responsibility
PAT	There is only one priority: to have a good minister of health	A good minister must accept the principle of patient-centred healthcare.

VII. Sources of information

Q.31 Which international or national sources of data or references do you frequently use to assess the quality of medication use and/or pharmaceutical care and/or pharmaceutical services?

	Sectors																	Total	Experts	
	Pharm	n	PH	n	MD	n	Pat	n	Nurs	n	Others	n	Unknown	n	%	n=58	%	n=6		
Not available	5.9%	1	12.5%	1	25.0%	1	28.6%	2	0.0%				0.0%		8.6%	5	0.0%			
Sources of data: see below	58.8%	10	62.5%	5	75.0%	3	14.3%	1	100.0%	4			5.9%	1	41.4%	24	33.3%	2		
Do not know	0.0%		0.0%		0.0%		14.3%	1	0.0%				0.0%		1.7%	1	0.0%			
No relevant answer	0.0%		12.5%	1	0.0%		28.6%	2	0.0%				0.0%		5.2%	3	33.3%	2		
Missing	35.3%	6	12.5%	1	0.0%		14.3%	1	0.0%		100%	1	94.1%	16	43.1%	25	33.3%	2		
		17		8		4		7		4	100%	1		17		58		6		

Sources of data cited by respondents:

- National pharmaceutical services
- National Ministry of Health
- International Pharmaceutical Association (FIP)
- World health Organization (WHO)
- Council of Europe
- (International) medical and pharmaceutical journals
- Food and Drug Administration (FDA)
- Irish Medicines Board (IMB)
- Institute for Safe Medicine Practice (ISMP)
- Bibliotheque Nationale de France (BNF)
- National Institute for Health and Clinical Excellence (NICE)
- Cochrane Library
- European Medicines Agency (EMA)

- National medicines regulatory authorities
- Pharmaceutical Care network Europe (<http://www.pcne.org/dokumenter/drug-related-problems/PCNE20classification20V5.01.pdf> for the classification of Drug-Related Problems)
- Pharmacopoeia
- National and local guidelines for drug use in different areas
- Europharm Forum
- Published reports from other countries
- Nordic collaboration.
- Epidemiological and drug utilisation data
- Österreichische Agentur für Gesundheit und Ernährungssicherheit (AGES)
- Interdisciplinary discussion with the doctor (he has relevant training)
- An errors report in dispensing of the University
- National surveys of hospitals, general practice, pharmacists and pharmaceutical industries

To assess the quality of the pharmaceutical care, respondents indicated the following sources of data:

	Sectors	n	PH	n	MD	n	Pat	n	Nurs	n	Others	n	Unknown	n	Total	Experts
	Pharm	1	12.5%	1	0.0%		28.6%	2	0.0%				0.0%		% n=58	% n=6
Not available	5.9%	1	12.5%	1	0.0%		28.6%	2	0.0%				0.0%		6.9%	0.0%
Sources of data: see below	58.8%	10	37.5%	3	50.0%	2	0.0%		50.0%	2			5.9%	1	31.0%	16.7%
Do not know	5.9%	1	0.0%		0.0%		0.0%		0.0%				0.0%		1.7%	0.0%
No relevant answer	0.0%		12.5%	1	0.0%		0.0%		25.0%	1			0.0%		3.4%	0.0%
Missing	29.4%	5	37.5%	3	50.0%	2	71.4%	5	25.0%	1	100%	1	94.1%	16	56.9%	83.3%
		17		8		4		7		4	100%	1		17	58	6

Sources of data cited by respondents:

- Community pharmacists/pharmacies: Health Inspectorate.
 - Therapeutical reference book, Martindale, Rote Liste
 - GPP document
 - www.delphicare.be, Vianova: scientific database that supports the pharmacist in applying the concept of pharmaceutical care
 - National pharmaceutical services
 - Peer reviewed articles and medical journals
 - International Pharmaceutical Association (FIP)
 - World Health Organisation (WHO)
 - Leading medical and pharmaceutical journals, national and international legislation and professional standards
 - Europharm Forum, Published reports from other countries
 - Nordic collaboration
 - Health outcome data of chronic diseases
 - Third Consensus of Granada
 - Bibliothèque Nationale de France (BNF)
 - Projects, presentations, lectures, posters - at congresses, seminars, lectures, etc
- Pharmacopoeia

To assess the quality of the pharmaceutical services, respondents answered the following sources of data:

	Sectors																Total	Experts
	Pharma	n	PH	n	MD	n	Pat	n	Nurs	n	Others	n	Unknown	n	%	n=58	%	n=6
Not available	11.8%	2	12.5%	1	0.0%		28.6%	2	0.0%				0.0%			5	0.0%	
Sources of data : see below	47.1%	8	25.0%	2	50.0%	2	0.0%		75.0%	3			5.9%	1	27.6%	16	16.7%	1
No relevant answer	0.0%		12.5%	1	0.0%		0.0%		0.0%				0.0%		1.7%	1	16.7%	1
Missing	41.2%	7	50.0%	4	50.0%	2	71.4%	5	25.0%	1	100%	1	94.1%	16	62.1%	36	66.7%	4
		17		8		4		7		4	100%	1		17		58		6

Sources of data cited by respondents:

- Community pharmacists/pharmacies; Pharmaceutical Department under the Ministry of Health (MOH)
- Therapeutical reference book
- National pharmaceutical services
- International Pharmaceutical Federation (FIP)
- World Health Organization (WHO)
- Ministry of Health
- Medical journals
- Pharmaceutical services: national and international legislation and professional standards, professional / regulatory / health service providers, particularly from countries such as the United Kingdom, Canada, Australia, New Zealand
- Google
- Europharm Forum, published reports from other countries, Nordic collaboration
- Drug consumption and morbidity data
- Clinical knowledge summaries
- Royal Pharmaceutical Society professional standards
- “Czech Chamber of Pharmacists arranges courses on special pharmaceutical services (asthma, diabetes, oncological diseases, etc) - outcomes are available on its Internet site” Literature, co-operation of specialists

VIII. Relevant stakeholders

Q.32 Are there already established co-operation mechanisms and structures between stakeholders in pharmaceutical care?

	Sectors															Total	Experts	
	Pharm	n	PH	n	MD	n	Pat	n	Nurs	n	Others	n	Unknown	Unknown	%	n=58	%	n=6
Yes	23.5%	4	50.0%	4	50.0%	2	28.6%	2	25.0%	1		1	0.0%		22.4%	13	16.7%	1
No	29.4%	5	0.0%		25.0%	1	28.6%	2	25.0%	1		1	0.0%		15.5%	9	66.7%	4
Partly, unofficial, limited, in development	23.5%	4	37.5%	3	25.0%	1	14.3%	1	0.0%				11.8%	2	19.0%	11	0.0%	
Do not know	0.0%		0.0%		0.0%		28.6%	2	50.0%	2		2	0.0%		6.9%	4	16.7%	1
Missing	23.5%	4	12.5%	1	0.0%		0.0%		0.0%		100%	1	88.2%	15	36.2%	21	0.0%	
		17		8		4		7		4	100%	1	17			58		6

Conclusions

Less than half of the respondents answered that there is established co-operation, at least partly (often informal, occasional, local, and still in the process of development due to the absence of a legal basis for such co-operation). One respondent said that such co-operation was stopped by the national Ministry of Health).

Q. 33 If co-operation has been established, which stakeholders are involved, how is the co-operation organised and in which fields (medication use, pharmaceutical care)?

Those respondents who reported about co-operation, replied to the question “which stakeholders are involved?” as follows:

Respondents mostly mentioned co-operation between two partners often within the pharmacy profession or between pharmacists and healthcare authorities. Multidisciplinary co-operation still seems only to exist in smaller projects. Examples of co-operation include:

- “Each month there is a meeting between the main national pharmacists’ associations, the Pharmaceutical department of the Ministry of Health, the Pharmacists’ Society (evaluation of the facilities for the providing pharmaceutical care in community pharmacies); Health Inspectorate (controlling function)”
- The Pharmaceutical Services and Patient Associations
- General practitioners and pharmacists State claims agency, hospitals, pharmacists, and health service management
- Health Service Executive, Health Information Quality Authority, State Claims Agency, and Department of Health
- Co-operation between university, pharmacy and medicine societies and pioneers in practice
- Authorities and Pharmacy Association
- Pharmacists, assistant of pharmacies, some doctors, and nurses
- Hospitals, general practitioners, pharmacists, and pharmaceutical industries
- The Clinical Pharmacy Association, College of Pharmacy Practice, and Guild of Health System Pharmacists
- Health insurance companies - project involving medication errors on prescriptions

Those respondents who reported about co-operation, replied to the question “which fields are covered by the co-operation?” as follows:

- “We discussed the guide for good pharmaceutical practices in dispensing (not yet published)”
- Legislation; monitoring; availability of services/premises
- Health services in pharmacies (chronic diseases such as diabetes, asthma, stopping smoking, etc)
- Rational use of medicines
- Repeat prescriptions, compliance, medication errors’ diagnosis, nursing care, pharmaceutical care

Those respondents who reported about co-operation, replied the question “how is co-operation organised?” as follows:

- Structural collaboration between authority and hospitals
- In the framework of legislation
- Local contracts
- At the moment, this field is being developed by the authorities
- On an individual basis
- Essentially informal - not government-sponsored
- Co-operation between insurance company and pharmacies ensured by different sources
- Seminar, conference, congress, training

Q. 34 Contact details of the respondents (Non-disclosure of contact details and identity of the respondents was included in the confidentiality arrangements for the survey, see Chapter 6)

IX. Feedback on the survey questionnaire

Q.35 Please give feed-back on the survey questionnaire.

The following feedback was received from respondents:

- “Why not anonymous? And with restricted access? This may lead to bias when assessing the questionnaires. Why not simply ask respondents to state their professional status?”
- “Some questions are difficult to interpret. Research is critical to making progress with this safety initiative.”
- “The questionnaire took a long time to complete; it was therefore unfortunately completed in a bit of a rush and with many interruptions. I hope I understood the questions correctly. I found the questions a bit difficult to understand at times (perhaps because of the interruptions). The issues were complex and really would have needed some thought and discussion here to bring out the salient points. Unfortunately in my country this topic is spread out over a number of areas so it is difficult to grasp and present a complete picture without discussion. We realise that this is a problem and it is something that we hope to be able to resolve in order to deal with this topic better in the future. Other countries may also have this problem?”
- “Very interesting and forward-looking.”
- “In the next survey we need to clarify the meaning of the terms of pharmaceutical care for all members. It’s important to provide an intercultural translation of these terms because they mean different things in different countries. Congratulations!”
- “I feel embarrassed to say the least that the questionnaire is in English, when 50% of the EU’s population speaks German. The organisation should be able to produce the questionnaire in German. EU scepticism may be fuelled. I would be very willing to answer the questions. (Note: At the respondent’s request, the questionnaire was answered in a telephone interview in German. The translations from German into English are not authorised translations. The answers to the survey questions were sent to the respondent for verification.)”
- “To have only 3 hours to answer is too much stress. Note: the report was entered manually from a faxed reply; comments on Q30 item 2 and Q 35 were translated from German into English (not an authorised translation).”
- “It would have been helpful to know from the outset how many questions there were and how long it would take to complete the survey.”
- “The main topic of the survey is not applicable in our country and therefore some of the points are difficult to answer.”
- “We would like to know the results of the survey throughout the EU.”
- “The question related to ‘pharmaceutical care’ requires a deeper evaluation. This matter involves national and local health competences. The concept of ‘pharmaceutical care’ is applied at local level in the hospital districts. For the points where we have given no answer, we would like to stress that by law most health competences are attributed to the local (regional) health authorities. However, the matter is so important that we are willing to contribute further with regard to our specific competences.”

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APPENDIX - SURVEY QUESTIONNAIRE

Welcome - Bienvenue

Dear Madam, Sir,

We would appreciate very much if you could complete this questionnaire at the latest by 20 December 2008.

Your information will contribute significantly to improving the quality of pharmaceutical care in Europe. In the frame of the activities of the European Directorate for the Quality of Medicines and HealthCare (EDQM), the Committee of Experts on quality and safety standards in pharmaceutical practices and pharmaceutical care is carrying out this survey.

After evaluation and discussion of the results by the responsible bodies, the participants will be informed about the aggregated outcome of the survey and further follow-up.

TECHNICAL INFORMATION

1. Before completing the questionnaire, please read statements 1, 2, 3 and 4 carefully.

You may print them out as reference when you draft your replies. You can find the statements in the questionnaire on pages 2, 4, and 6 (pages, see bar, top right).

You can find a glossary of terms on questionnaire page 2.

2. The survey questionnaire is available in English and French. Please use English or French for your replies.

3. Please complete this questionnaire "on line" by

- replying either by ticking suggested options (e.g. Yes or No) or by inserting text into text boxes,
- completing the questions of chapter by chapter (chapter designation, see title bars and top right).

After you have completed the questions on one page, click on "Next" to move on to the next page and "Prev" to go back.

If you would like to leave the questionnaire, just click on "Exit this survey". Your answers will be saved.

You can print out the questionnaire. You can revise your replies until 30 December 2008 by re-entering revised information. Please use the same link to the questionnaire.

CONTACT

For further questions, please contact

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- the EDQM-DBO Secretariat, Sabine Walser; E-mail: sabine.walser@edqm.eu,

CONFIDENTIALITY

All individual information communicated in the course of this survey to the Secretariat will remain strictly confidential. No communication or publication will be made without the prior approval of all participating States.

Happy data entering !

Madame, Monsieur,

Nous vous remercions de remplir ce questionnaire au plus tard le 20 décembre 2008.

Les informations incluses dans votre réponse seront très utiles à l'amélioration du suivi pharmaceutique en Europe. Parmi les activités menées par la Direction Européenne de la Qualité du Médicament & Soins de Santé (DEQM), le Comité d'experts sur les normes de qualité et sécurité relatives aux pratiques et suivi pharmaceutiques est chargé de la présente enquête.

Après évaluation et discussion des résultats par les organes concernés, les participants seront tenus au courant des résultats et du suivi à donner.

INFORMATIONS TECHNIQUES

1. Avant de compléter le questionnaire, merci de lire attentivement les remarques 1, 2, 3 et 4.

Vous pouvez les imprimer comme textes de référence quand vous répondrez aux questions. Vous pouvez trouver les remarques sur les pages 2,4 et 6 du questionnaire ("pages", voir en-tête, en haut à droite).

Vous pouvez trouver un glossaire des termes sur la page 2 du questionnaire.

2. Le questionnaire est disponible en anglais et en français. Veuillez répondre en anglais ou en français.

3. Merci de compléter le questionnaire en ligne :

- répondez en cliquant sur les boutons (par exemple Oui ou Non) ou remplissez les champs,
- remplissez page par page (en-tête, en haut à droite).

Après avoir rempli les questions d'une page, cliquez sur « NEXT » afin d'avancer vers la page suivante, cliquez sur « PREV » afin de revenir sur la page précédente.

Si vous voulez quitter, cliquez sur « EXIT this survey ». Vos réponses seront sauvegardées.

Vous pouvez imprimer le questionnaire rempli. Vous pouvez modifier vos réponses jusqu'au 30 décembre 2008 en réintroduisant une information révisée. Veuillez utiliser le même lien vers le questionnaire.

CONTACT

Pour toute question, contactez

- Nico KIJLSTRA, coordinateur du projet « Indicateurs de la qualité et de la sécurité du suivi pharmaceutique en Europe » ; e-mail : nb.kijlstra@igz.nl
- le Secrétariat du SBO (DEQM), Sabine Walser, e-mail : sabine.walser@edqm.eu,

CONFIDENTIALITE

Toutes les informations individuelles communiquées au Secrétariat au cours de cette enquête demeureront strictement confidentielles. Aucune communication ou publication ne sera faite sans l'approbation préalable de tous les Etats membres participants.

Happy data entering!

Understanding the concept Pharmaceutical Care / Comprendre le concept de Suivi pharmaceutique

Please read statements 1 and 2 carefully before completing the questionnaire. You may print them out as reference when you draft your replies.

Statement 1

Pharmaceutical care is the responsible provision of drug therapy for the purpose of achieving definite outcomes that improve a patient's quality of life. These outcomes are

- cure of a disease;
- elimination or reduction of a patients' symptomatology;
- arresting or slowing of a disease process; or
- preventing a disease or symptomatology.

Pharmaceutical care involves the process that a pharmacist co-operates with the patient and healthcare professionals in designing, implementing, and monitoring a therapeutic plan that will produce specific therapeutic outcomes for the patient. This in turn involves three major functions:

- identifying potential and actual drug-related problems;
- resolving actual drug-related problems; and
- preventing drug-related problems.

Pharmaceutical care is a necessary element of health care, and should be integrated with other elements. Pharmaceutical care is, however, provided for the direct benefit of the patient, and the pharmacist is responsible directly to the patient for the quality of that care. The fundamental relationship in pharmaceutical care is a mutually beneficial exchange in which the patient grants authority to the provider and the provider gives competence and commitment (accept responsibility) to the patient. These fundamental goals, processes, and relationships of pharmaceutical care exist regardless of practice setting and of professional background.

Helper, D.D. & Strand, L.M. Opportunities and Responsibilities in Pharmaceutical Care, Am.J. Pharm. Educ., 53, 7S-15S(1989)

Statement 2

The term "pharmaceutical care" was chosen by the Committee of Experts Quality and Safety Standards in Pharmaceutical Practices and Pharmaceutical Care (CD-P-PH/PC) as a working definition for the concept explained in box 1, for the purpose of this survey questionnaire. The Committee of Experts acknowledges that both the concept and term may not be used in all States parties to the Convention on the Elaboration of a European Pharmacopoeia to the same extent.

Glossary

Drug therapy: for the purpose of this questionnaire, this is understood as medicine therapy.

Indicator: A summary measure that aims to describe, in a few numbers as much detail as possible about a system, to help understand, compare, predict, improve, and innovate¹.

Medication review:

A systematic assessment of the medication use of an individual, carried out by a pharmacist in close cooperation with both the prescriber and the patient.

Outcome: An outcome is a measurable change in health status, sometimes attributable to a risk factor or an earlier intervention².

Proxy of outcome

A measure which represents a specific outcome that cannot be measured directly.

¹NHS Institute for Innovation and Improvement: The Good Indicators Guide: Understanding how to use indicators

www.institute.nhs.uk

Veuillez lire attentivement les remarques 1 et 2 avant de compléter le questionnaire (voir page suivante). Vous pouvez imprimer les remarques et les utiliser comme des textes de références quand vous répondez aux questions.

Remarque 1

Le suivi pharmaceutique est la mise en oeuvre de différents processus permettant une dispensation responsable du traitement médicamenteux jusqu'à l'obtention de résultats tangibles qui améliorent la qualité de vie du patient. Ce résultat peut être

- la guérison d'une maladie ;
- l'élimination ou la réduction des symptômes d'un patient ;
- l'arrêt ou le ralentissement de la progression d'une maladie ; ou
- la prévention d'une maladie ou de symptômes.

Le suivi pharmaceutique implique une coopération du pharmacien avec le patient et les professionnels de santé pour élaborer, appliquer et suivre une conduite thérapeutique qui produira des résultats thérapeutiques spécifiques chez le patient. Il implique trois fonctions majeures :

- identifier des problèmes médicamenteux existants ou potentiels ;
- résoudre les problèmes médicamenteux existants ; et
- prévenir tout problème médicamenteux.

Le suivi pharmaceutique constitue un des éléments nécessaires des soins de santé. Il vise à bénéficier directement au patient, le pharmacien étant le responsable direct de la qualité de ce suivi envers le patient. La relation fondamentale sur laquelle se base le suivi pharmaceutique est un échange mutuellement bénéfique entre le patient qui confère un pouvoir de décision au fournisseur et le fournisseur qui apporte compétence et engagement (prise de responsabilité) au patient. Ces objectifs, processus et relations sont fondamentaux pour le suivi pharmaceutique, quels que soient l'environnement ou la profession de l'intervenant.

Helper, D.D. & Strand, L.M. Opportunities and Responsibilities in Pharmaceutical Care, Am.J. Pharm. Educ., 53, 7S-15S(1989)

Remarque 2

Le terme « suivi pharmaceutique » a été choisi par le Comité d'Experts sur les normes de qualité et de sécurité relatives à la pratique et au suivi pharmaceutiques (CD-P-PH/PC) et sert de définition de travail au concept expliqué dans l'encart 1, dans le cadre du présent questionnaire. Le Comité d'Experts est conscient que le concept, tout comme le terme, peuvent être plus ou moins répandus ou utilisés au sein des états parties à la Convention sur l'élaboration d'une Pharmacopée Européenne.

Glossaire

Indicateur : une mesure sommaire visant, en quelques données chiffrées, à décrire de manière aussi détaillée que possible un système, pour mieux le comprendre, le comparer, l'anticiper, l'améliorer, et l'innover².

Analyse de médication :

Une évaluation systématique des médicaments utilisés par un individu, menée par un pharmacien en étroite coopération avec le prescripteur et le patient.

Résultat : un changement quantifiable de l'état de santé, parfois attribuable à un facteur de risque ou à une intervention antérieure¹.

Résultat de substitution :

Une mesure qui représente un résultat spécifique ne pouvant être directement mesuré.

¹NHS Institute for Innovation and Improvement: The Good Indicators Guide: Understanding how to use indicators
www.institute.nhs.uk

***1. Do you think the concept of “pharmaceutical care” is known in your country?
Pensez-vous que le concept de « suivi pharmaceutique » est connu dans votre pays ?**

***2. Do you think the term “pharmaceutical care”, statement 1, is widely used in your country?
Pensez-vous que l’emploi du terme « suivi pharmaceutique », remarque 1, est répandu dans votre pays ?**

***3. The concept of pharmaceutical care, statement 1, focuses on the activities of the pharmacist. Do you think the concept is sufficiently comprehensive to take account of the roles of all stakeholders of medicine therapy?
Le concept de suivi pharmaceutique remarque 1, met l’accent sur les activités du pharmacien. Pensez-vous que la prise en compte de tous les intervenants dans le traitement pharmacologique soit assez clairement comprise dans ce concept ?**

***4. Which alternative terms are used for the concept of “pharmaceutical care” in your country?
Quels autres termes expriment le concept de « suivi pharmaceutique » dans votre pays ?**

***5. In your view, which are the professions involved in the activities described in the concept of pharmaceutical care, statement 1, 2nd paragraph?
Selon vous, quelles sont les professions impliquées dans les activités décrites dans le concept du suivi pharmaceutique encart 1, 2^e paragraphe ?**

Use of the concept Pharmaceutical care / Utilisation du concept de Suivi pharmaceutique

***6. To what extent is the concept of pharmaceutical care, statement 1, applied practically by representatives of the following professions?**

Dans quelle mesure le concept de suivi pharmaceutique remarque 1 est-il appliqué en pratique par les représentants des professions suivantes ?

	concept not applied practically/ pas appliqué en pratique	concept considered desirable, few apply/considere desirable	concept common practice/courant	concept minimum standard, some extend/ Appartient aux normes de base
Pharmacists/ Pharmaciens	☐	☐	☐	☐
Doctors/Médecins	☐	☐	☐	☐
Nurses/Infirmiers	☐	☐	☐	☐
Other stakeholders/ Autres intervenants, see comments	☐	☐	☐	☐
Comments. General/ Other stakeholders, pls specify / spécifier les autres intervenants				

***7. How would you describe the role in pharmaceutical care of**
Comment décririez-vous le rôle, dans le suivi pharmaceutique, des

Pharmacists/Pharmaciens	
Doctors/Médecins	
Nurses/Infirmiers	
Other Healthcareprofessionals/ Autres professionnels de santé	
Patients/Patients	
Carers/Chargés des soins	

***8. In your views, which outcome has the application of the concept of pharmaceutical care, statement 1, on the patients' medicine therapy?**

Pensez-vous que l'influence de mise en œuvre du concept de suivi pharmaceutique remarque 1 sur le résultat du traitement médicamenteux d'un patient est

☐ Good outcome/bonne ☐ No outcome/nulle ☐ Bad outcome/mauvaise ☐ Do not know/Ne sais pas

9. Please describe the barriers for implementing in practice the concept of pharmaceutical care, statement 1, in your country?

Veillez décrire les obstacles à la mise en application pratique du concept de suivi pharmaceutique remarque 1 dans votre pays ?

***10. Do health care professionals share patients' records in order to promote the use of the concept?**

Afin de promouvoir l'utilisation du concept, les professionnels de santé partagent-ils les dossiers des patients ?

☐ Yes/Oui
☐ No/Non
☐ Do not know/ Ne sais pas

If yes, how do health care professionals share patients' records/Dans l'affirmative, comment les dossiers des patients sont-ils partagés entre les professionnels de santé ?

**11. Is the concept of pharmaceutical care, statement 1, actively implemented or accepted as part of professional practice by the health care professional bodies in your country?
Le concept de suivi pharmaceutique remarque 1 est-il activement appliqué ou accepté comme faisant partie de la pratique professionnelle par les ordres professionnels dans votre pays ?**

- ☐ Implemented/mis en oeuvre
☐ Accepted/accepté
☐ Unfamiliar/méconnu

If implemented, please describe how/Si le concept est appliqué, veuillez indiquer comment :

**12. What is the priority of the concept of pharmaceutical care, statement 1, in your government policy on medication therapy?
Quelle priorité est accordée au concept de suivi pharmaceutique remarque 1 dans la politique de votre gouvernement en matière de traitement pharmacologique ?**

- ☐ high priority/priorité élevée
☐ middle priority/priorité moyenne
☐ low priority/priorité faible
☐ under consideration for setting future priorities/en cours d'évaluation
☐ no priority/priorité nulle
☐ Do not know/Ne sais pas

Please specify reasons for the priority setting/Pourriez-vous indiquer les motifs de cette priorité ?

***13. Is there a legal basis, health strategy or policy document for the implementation of the concept of pharmaceutical care, statement 1, in your country?
Existe-t-il une base juridique, une stratégie ou un document d'orientation en matière de santé, concernant la mise en application du concept de suivi pharmaceutique remarque 1 dans votre pays ?**

- ☐ Yes/ Oui
☐ No/Non
☐ Do not know/Ne sais pas

Please include web link to document or send to sabine.walser@edqm.eu/Inserez le lien web vers ce document ou envoyez-le à sabine.walser@edqm.eu/ Fax + 33 3 88 41 27 71

14. In your country, are there contracts between health professionals and employers, insurers and health service providers to implement the concept of pharmaceutical care, statement 1?

Dans votre pays, existe-t-il des contrats entre professionnels de santé d'une part et employeurs, assureurs ou fournisseurs de services de santé d'autre part concernant la mise en application du concept de suivi pharmaceutique remarque 1 ?

- ☐ Yes/ Oui
☐ No/ Non
☐ Do not know/ Ne sais pas

Please include web link to document or send to sabine.walser@edqm.eu/ Inserez le lien web vers ce document ou envoyez-le à sabine.walser@edqm.eu/ Fax + 33 3 88 41 27 71

Management of medicine therapy - a cycle / Gestion du traitement médicamenteux - un cycle

Statement 3

Being a part of pharmaceutical care, the management of medication and use is a cycle of actions: from the decision to use medication, drawing up a treatment plan, implementing, monitoring and managing, reassessing medicine therapy from diagnosis to cure, throughout maintenance of (chronic) therapy or stopping of medicine therapy. The different processes in the medication use chain are part of different quality cycles: self-medication,

prescription, (preparation), dispensing, administration, changing or stopping medication, continuous evaluation of the medication effect. The overall effect of the medication depends on the strength of the links in the chain, and on whether and how well they are connected. To be able to assess the value or the quality of medication use, one must also assess the outcomes, not (only) the quality of the various activities involved.

Please read statement 3 carefully before completing the questionnaire. You may print it out as reference when you draft your replies.

Remarque 3

Faisant partie du suivi pharmaceutique, la gestion du traitement médicamenteux et de l'usage des médicaments constitue une chaîne d'activités et de processus consécutifs ou parallèles (un cycle d'actions), comme la décision du recours à une médication, la planification d'un traitement, sa mise en application, son suivi et sa gestion, la réévaluation du traitement pharmacologique par rapport au diagnostic, pour aboutir à la guérison, à travers le maintien d'un traitement chronique ou l'arrêt du traitement médicamenteux. Les différents processus constituant la chaîne de la pratique médicamenteuse appartiennent à différents cycles de qualité : l'automédication, la prescription, (la préparation), la dispensation, l'administration, le changement ou l'arrêt de la médication, l'évaluation continue de l'effet de la médication. L'effet global de la médication dépend de la résistance des maillons de la chaîne et de leur bon assemblage. Pour pouvoir évaluer la qualité de l'usage d'une médication, il convient également d'évaluer les résultats, et non pas seulement la qualité des diverses activités impliquées.

Veuillez lire attentivement les informations figurant dans la remarque 3 avant de compléter le questionnaire .

***15. How do you rank the importance of the management of the medicine therapy and medication chain?**

Comment qualifiez-vous l'importance de la gestion de la chaîne / du processus du traitement médicamenteux et de l'usage des médicaments ?

- ☐ High/Elevée
- ☐ Medium/ Moyenne
- ☐ Low/Faible
- ☐ None/Nulle
- ☐ Do not know/ Ne sais pas

***16. Is the quality of the management of the medicine therapy and medication chain in your country being measured on a routine basis?**

La qualité de la gestion de la chaîne / du processus du traitement médicamenteux et de l'usage des médicaments dans votre pays est-elle mesurée en routine ?

- ☐ Yes/Oui
- ☐ No/Non
- ☐ Do not know/ Ne sais pas

***17. If the reply to question 17 was YES, how is the quality of the management of medicine therapy and medication chain measured?**
Si la réponse a la question 17 a été OUI, comment est-elle mesurée ?

Note:

Structures: premises, equipment, registries, forms, standards.

Data on processes: the extent to which protocols and standards are used, how safety signals, adverse drug events or complaints are dealt with.

Health outcome indicators or proxies: mortality and morbidity rates, (re-)hospitalisation rate, incidence of serious side effects, inappropriate discontinuation, see also Glossary

Existence of structures/ Existence de structures	
Data on processes/ Données sur les processus	
Health outcome indicators or proxies of outcome indicators/ Indicateurs de résultats ou indicateurs de substitution	
Economic data/ expenditure/Données économiques / dépenses	
Other/Autres	
Do not know/Ne sais pas	

Pharmaceutical care & safe use / Suivi pharmaceutique & sécurité

Good pharmaceutical care includes the safe use of medicines. However, when medication errors happen they are potentially harmful. Medication safety is important for the proper use of medicines, whether they are prescribed or taken as self-medication.

Un suivi pharmaceutique approprié comprend la sécurité de la médication. Quand se produisent des erreurs de médication, elles peuvent être dangereuses. La sécurité de la médication est importante pour un bon usage des médicaments, qu'ils soient prescrits ou auto-administrés.

**18. How do you rank the importance of the safe use of medicines for overall patient safety?
Comment qualifiez-vous l'importance de la sécurité dans l'emploi d'une médication sur la sécurité globale du patient ?**

- ☐ High/Elevée
- ☐ Medium/Moyenne
- ☐ Low/Faible
- ☐ None/Nulle
- ☐ Do not know/Ne sais pas

***19. In which areas of the national healthcare system is patient safety measured in your country?
Dans quels domaines du système national de santé de votre pays est mesurée la sécurité du patient ?**

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**20. Is the safety of medication use measured in your country?
La sécurité dans l'usage d'une médication est-elle mesurée dans votre pays ?**

- ☐ Yes/Oui
- ☐ No/Non
- ☐ Do not know/Ne sais pas

***21. If the reply to question 21 was YES, how is the quality of the management of medicine therapy and medication chain measured?**
Si la réponse à la question 21 a été OUI, comment la sécurité dans l’usage d’une médication est-elle mesurée ?

Note:

Structures: premises, equipment, registries, forms, standards.

Data on processes: the extent to which protocols and standards are used, how safety signals, adverse drug events or complaints are dealt with.

Health outcome indicators or proxies: mortality and morbidity rates, (re-)hospitalisation rate, incidence of serious side effects, inappropriate discontinuation, see also Glossary

Existence of structures/ Existence de structures	
Data on processes/ Données sur les processus	
Health outcome indicators or proxies of outcome indicators/ Indicateurs de résultats ou indicateurs de substitution	
E conomic data/ expenditure/Données économiques / dépenses	
Other/Autres	
Do not know/Ne sais pas	

***22. How could the measurement of patient safety be improved?**
Comment la sécurité du patient pourrait-elle mieux mesurée ?

Patient participation-key to good outcomes / Participation du patient-essentiel aux bons résultats

Statement 4

Good outcomes from medication are best achieved when the patient and the healthcare professional agree jointly on a treatment plan, which takes into account the patient's beliefs and attitudes. This can be influenced by providing adequate information to underpin the sense of self-management. Tools for measuring patients' experiences are available in several healthcare areas.

Please read statement 4 carefully before completing the questionnaire. You may print it out as reference when you draft your replies.

Remarque 4

Une médication donne de meilleurs résultats si le patient et le professionnel de santé acceptent ensemble un plan de traitement qui tient compte des convictions et des attitudes du patient. Une information adéquate étayant le sens de l'autogestion peut avoir une bonne influence sur ces résultats. Des outils permettant de mesurer les expériences des patients existent dans plusieurs domaines des soins de santé.

Veuillez lire attentivement les informations figurant dans la remarque 4 avant de compléter le questionnaire.

***23. How do you rank the importance of joint decision-making between the patient and the health professional for good outcomes?**

Comment qualifiez-vous l'importance d'une prise de décision commune entre le patient et le professionnel de santé dans l'obtention de bons résultats ?

- ☐ High/Elevée
- ☐ Medium/Moyenne
- ☐ Low/Faible
- ☐ None/Nulle
- ☐ Do not know/Ne sais pas

Please indicate areas for improvement, if applicable/quels domaines éventuels pourraient être améliorés ?

***24. How do you measure the quality and extent of joint decision-making in medication therapy?**

Comment mesurez-vous la qualité et l'étendue d'une prise de décision commune en matière de traitement pharmacologique ?

***25. If the quality and extent of joint decision-making in medication therapy was NOT measured in your country, how would YOU propose measuring it?**

Si une mesure de la sorte N'existe PAS dans votre pays, comment procéderiez-VOUS ?

***26. How do you assess the impact of joint decision-making?**

Comment évaluez-vous l'impact d'une prise de décision commune ?

***27. If the impact of joint decision-making in medication therapy was NOT assessed in your country, how would YOU propose assessing it?**

Si une évaluation de la sorte N'existe PAS dans votre pays, comment procéderiez-VOUS ?

Ensuring best possible outcome of medication / Meilleur effet possible à atteindre

***28. What systems do you have in place to ensure good, clinically effective outcomes of medication at population level?**
Quels systèmes sont en place pour garantir de bons (efficaces du point de vue clinique) résultats de médication au niveau de la population ?

--

***29. Please describe 3 priorities for your country in the fields of safety and quality of pharmaceutical care:**
Veillez décrire 3 objectifs prioritaires dans votre pays en matière de sécurité et de qualité du suivi pharmaceutique :

1.	
2.	
3.	
Comments:	

***30. Please describe how the 3 above priorities in the fields of safety and quality of pharmaceutical care are measured and assessed:**
Décrivez comment est mesuré et évalué chacun des 3 objectifs susmentionnés :

1.	
2.	
3.	
Comments:	

Sources of information in pharmaceutical care / Sources d'information

***31. Which international or national sources of data or references do you frequently use to assess the quality of medication use and/or pharmaceutical care and/or pharmaceutical services?**

Quelles sources de données ou références internationales ou nationales utilisez-vous fréquemment pour évaluer la qualité de l'usage d'une médication / du suivi et des services pharmaceutiques ?

Quality of medication use/
qualité de l'usage d'une
médication

Pharmaceutical Care/suivi
pharmaceutique

Pharmaceutical Services/
services pharmaceutiques

Co-operation: stakeholders in pharmaceutical care / Coopération :
Intervenants concernés

***32. Are there already established co-operation mechanisms and structures between stakeholders in pharmaceutical care?**
Existe-t-il déjà des mécanismes et structures établis de coopération entre l'ensemble ou une partie des intervenants du suivi pharmaceutique ?

***33. If co-operation has been established, which stakeholders are involved, how is the co-operation organised and in which fields (medication use? pharmaceutical care?)**
Dans l'affirmative, quels sont les intervenants concernés, comment la coopération est-elle organisée et dans quel(s) domaine(s) de l'usage d'une médication / du suivi pharmaceutique ?

Which stakeholders are involved? Quels intervenants concernés?	
Which fields are covered by the cooperation (see question 34)? Quels domaines couverts (voir question 34) ?	
How is cooperation organised? Comment la coopération est-elle organisée?	

Your contact details

34. Please enter the contact details of the person who has replied to the questionnaire.

Note: if you have entered the replies on behalf of another person, please add the contact *details* of that person.

Merci d'insérer les coordonnées de la personne ayant répondu au questionnaire.

Note: si vous avez répondu au nom d'une autre personne, veuillez indiquer le nom de cette personne.

First Name	
Last Name	
Job title	
Organisation	
Address	
City	
Code Postal	
Member State	
Tel.:	
Fax:	
E-mail	

Expression of thanks / Remerciements

We would like to thank you very much for your time spent on completing the questionnaire!
Nous souhaitons vous remercier vivement pour le temps passé à remplir ce questionnaire!

35. Please let us know if you have general comments on this survey/ Commentaires généraux sur cette enquête.

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PHARMACEUTICAL CARE

Where do we stand - Where should we go?

Pharmaceutical care is a necessary element of healthcare.

According to the definition of Helper and Strand (1989), pharmaceutical care is the responsible provision of medicine therapy for the purpose of a definite outcome that improves a patient's quality of life. Pharmaceutical care is based on a relationship between the patient and the healthcare providers who accept responsibility for the patient. This concept implies the active participation of the patient in medicine therapy decisions, the cooperation of healthcare providers across disciplines, and gives priority to the direct benefit of the patient.

The Committee of Experts on quality and safety standards for pharmaceutical practices and pharmaceutical care contributes to the mission of the EDQM which is to promote access to good quality healthcare. It is carrying out programmes and developing policies, giving priority to the needs of patients and the social and ethical values of healthcare.

Within its programme of activities, the above Committee of Experts studied in 2008/2009 the current awareness and understanding of the concept of pharmaceutical care in Europe, approaches for the quality assessment of pharmaceutical care, the extent to which pharmaceutical care is implemented in practice, and the competences and skills required.

This report analyses and presents conclusions of 58 replies from national public health authorities, doctors', pharmacists', nurses', and patients' associations from 17 countries.

It was found that pharmaceutical care is increasingly being considered an important goal but not yet implemented in practice due to varying awareness and education among healthcare providers, and inadequate cooperation among healthcare providers.

The quality of management of medicine therapy, outcomes in the patient, and the safe use of medicines are not measured on a routine basis in most countries in Europe.

Only a few countries have a legal basis for the implementation of pharmaceutical care or have regular contacts between healthcare professionals and insurers for the purpose of implementation of pharmaceutical care.

The European Directorate for the Quality of Medicines & HealthCare (EDQM) is a Directorate of the Council of Europe, an international organisation founded in 1949 which to date has a membership of 47 states in Europe. The primary aim of the Council of Europe is to create a common democratic and legal area throughout the whole of the continent, ensuring respect for its fundamental values: human rights, democracy and the rule of law.

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© 2009 Council of Europe
ISBN 978-92-871-6712-5